



Evaluation and Service Planning

Resident Care Manual

1.1 Assisted Living OverviewRevised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Merrill Gardens seeks to maintain a residential atmosphere for our residents. We accept and retain residents who have health conditions that fall within the limits of the licensure in the state of operation, and whom our Communities will best serve.

Definitions

Assisted Living Services:

Team members providing the resident living in a licensed apartment with assistance with activities of daily living, such as bathing, dressing, grooming, and medications based on the evaluation and service plan.

Guidelines

This policy describes the admission criteria, the evaluation and service plan process, and fee structures. Enhancing resident independence and supporting ongoing individual choice are important goals for Merrill Gardens. We view Assisted Living as a wellness program with the goal of helping a resident maintain the highest level of function for as long as possible.

Certain communities will offer a care option for residents whose memory loss has become progressively worse and who need a more secure environment and specialized program of care. Merrill Gardens refers to these specialized environments as "The Garden House."

Evaluation

Each resident will undergo an evaluation prior to acceptance for residency to determine the level of care and services they will need. The evaluation may utilize:

- Information provided by the primary care provider (Merrill Gardens Medical Health Statement, history and physical, and/or state specific form)
- Observations made, and initial information gathered by Community team members (touring process, responsible party, family, and/or referral sources)

The comprehensive evaluation will be conducted by a licensed nurse or qualified assessor using Merrill Gardens' evaluation and service planning software.

General Health

Merrill Gardens provides assisted living services, but is not a skilled nursing facility. Prospective residents must meet acceptance and retention criteria for admission to the Community. Services are packaged in levels of care that are priced according to the resident's needs, and the demands of the local

marketplace. The licensed nurse or designee completes an initial evaluation and service plan prior to move in, followed by a review within the first thirty (30) days of residency. The evaluation and service plan are reviewed every six months, or upon change in condition.

WA: *The full evaluation must be completed by a qualified assessor within five (5) days prior to, or within fourteen (14) days after move in. The Service Plan is to be completed within the first thirty (30) days of residency.*

In order to qualify and remain in a Merrill Gardens Community, the resident must:

- Be free of health conditions specifically prohibited by state regulation, and/or Community policy as described in this manual.
- Have level of independence to perform daily living activities. Supportive assistance is acceptable.
- Must not require 24-hour skilled nursing care. State regulatory requirements will apply.

Have a licensed physician or nurse practitioner complete the Medical Health Statement and/or provide a History and Physical, and/or state form. Where applicable, a Tuberculosis (TB) test may be required prior to move-in. Health exams should be completed prior to move-in. Health exams are only valid if done within sixty (60) days prior to move in. Residents receiving assisted living services must have a local primary physician or health care practitioner (as allowed by regulation) designated at all times.

CA: *Health Exams are valid if done within the past year.*

SC: *The physical exam must be completed within thirty days prior to admission.*

Garden House Principles

The Garden House care philosophy is based upon the following principles:

PROMOTE THE HIGHEST FUNCTION AND QUALITY OF LIFE

- We focus on individuals' abilities rather than disabilities.
- To improve residents' quality of life, we try to meet their psychological, social, physical and spiritual needs.

RESPECT THE INDIVIDUAL

- We recognize that residents with dementia maintain the need for privacy, independence, accomplishment, self-identity and self-determination.
- We respect residents' values, unique life experiences and histories.
- We recognize that residents' needs fluctuate throughout the disease process.
- In addition to specific "resident rights" outlined in state regulations, we advocate the following rights for all residents:
 - Freedom from chemical and physical restraints
 - Freedom to move without being confined, including space outdoors
 - Privacy and respect for the resident and their personal belongings

- Respect for each individual's valued experiences and activities

ENCOURAGE FAMILY INVOLVEMENT

- We know that, in addition to the individual who is diagnosed with dementia, families are greatly affected, and need care and support as well.
- We encourage families to contribute to their loved one's care and to the care planning process.
- We provide resources and support to families to assist them in coping with the disease.

PROVIDE A CONTINUUM OF CARE

- We provide care throughout the early and middle stages of dementia and coordinate care options that may allow residents to remain living in our communities until they die.
- For residents who are in the end stages of dementia, we consider the resident's safety, state regulations and the wishes of the family in identifying the appropriate care options.

PROVIDE A FLEXIBLE PROGRAM OF ACTIVITIES THAT INCLUDES TEAM MEMBERS

- All team members who work in our Garden Houses are facilitators in a program designed to maximize the abilities of each resident.
- We focus on the remaining abilities of the resident and design activities with those abilities in mind.
- To enhance function and well-being, we implement a program of activities that includes social interactions, everyday events, exercise, nutrition and opportunities for choice.
- Each Garden House has a planned, written program of activities; however, we adjust the delivery of services to the unique needs of the residents.

DEVELOP A POSITIVE, SUPPORTIVE ENVIRONMENT

- We create environments where residents can feel safe and comfortable, and where families can feel welcome.
- Our Garden Houses incorporate safety features, cues and way-finding assistance to support residents' independence and freedom of movement within.

Assisted Living Efficiencies

PROVIDING EFFICIENT SERVICES

Staffing is based on resident service needs. State laws may dictate specific staffing ratios at different times of the day, or given building layout. Some communities may utilize the Universal Worker model, where additional team members are trained and responsible for caregiving duties.

Trained Caregivers specially assigned to the Assisted Living team have two job responsibilities:

- Caring for Assisted Living residents, including supportive services.
- Observing and assisting other residents as needed.

The converse is true for team members outside the Assisted Living department. The entire Community team should view, as part of their jobs, observing and reporting any changes in conditions or concerns to their supervisor or the General Manager.

Assisted Living Fee Structure

The service plan, medical information and participation of the resident and/or responsible party are all used to develop a service plan customized to meet the resident's needs.

CHARGING ACCURATELY AND APPROPRIATELY FOR SERVICES.

Merrill Gardens provides high quality services at a fair price. In order to charge appropriately, our fees are based on levels of care. Residents will be charged for the service level determined by their evaluation and service plan. The resident is re-evaluated on a routine basis and with change in condition.

Care fees are billed in advance and are refundable based on the Residency Agreement. If a resident's needs change mid-month, the charge is adjusted on the date that revised services begin and pro-rated accordingly. The change(s) in service will appear on the next billing statement.

CA: The resident or their representative will receive written notice of a care fee increase within two (2) business days of us providing services at the new level of care.

OTHER SERVICES

In addition to the levels of care, residents may add other services, provided they are offered by the Community, and are applicable. Additional available services are listed in the Residency Agreement and may include:

- Personal laundry
- Additional housekeeping
- Additional linen service
- Personal errand services

1.2 Assisted Living Evaluation and Service Planning

.....Revised 10/24/2018 Reviewed 10/24/2018

Policy Purpose

Merrill Gardens utilizes a team approach to evaluate a resident and develop a Service Plan for Assisted Living (AL) services. The team includes community staff, the resident and/or responsible party, and the resident's physician or health care provider. An Evaluation and Service Plan incorporates the resident's care needs, capabilities and preferences. Through this evaluation, the community plans services with a goal of meeting the needs and expectations of assisted living residents. Evaluations for all residents in licensed apartments will be completed according to the following timeline(s):

Type	Frequency
Initial Evaluation	Prior to move-in (new residents)
Post-move-in evaluation	Thirty (30) days from the date of move-in or per state requirements
Semi-Annual	Every six (6) months
Change of Condition	As needed when changes are observed or reported
State-specific	Based on state regulatory requirements

AZ: The service plan must be completed within 14 calendar days of move in. The Service Plan must be reviewed and updated no later than 14 calendar days after a significant change in condition and as follows: at least every 12 months for supervisory services, at least every 6 months for personal care services and at least every 3 months for directed care services.

GA: Post Move-in evaluation must be completed within 14 days of the date of move in.

SC: A written assessment of the resident in accordance with Section 101.H shall be conducted by a direct care staff member as evidenced by his or her signature and date within a time-period determined by the facility, but no later than 72 hours after admission.

WA: The full assessment must be completed within 14 days of move in and at least annually.

Definitions

Assisted Living Services

Team members providing the resident living in a licensed apartment with assistance with activities of daily living, such as bathing, dressing, grooming, and medications based on the evaluation and service plan.

Electronic Health record (EHR)

Software used to document care Evaluations and Service Plan information.

WA: Qualified Assessor-The person responsible for completing a preadmission assessment of a prospective resident:

- (1) Has a master's degree in social services, human services, behavioral sciences or an allied field and two years social service experience working with adults who have functional or cognitive disabilities; or*
- (2) Has a bachelor's degree in social services, human services, behavioral sciences, or an allied field and three years social service experience working with adults who have functional or cognitive disabilities; or*
- (3) Has a valid Washington state license to practice nursing, in accordance with chapters 18.79 RCW and 246-840 WAC; or*
- (4) Is a physician with a valid state license to practice medicine; or*
- (5) Has three years of successful experience acquired prior to September 1, 2004, assessing prospective and current assisted living facility residents in a setting licensed by a state agency for the care of vulnerable adults, such as a nursing home, assisted living facility, or adult family home, or a setting having a contract with a recognized social service agency for the provision of care to vulnerable adults, such as supported living.*

Guidelines

Assisted Living Program

Merrill Gardens promotes independence among our residents. We may provide assisted living services within the scope of our license for those residents evaluated as appropriate for residency. All prospective residents of a licensed community are evaluated prior to acceptance as a resident in the community. The evaluation will review their functional and health related conditions and determine whether there are service needs. This policy also applies to current residents returning to the community following a change in condition such as rehabilitation or hospitalization.

Evaluation Resources

When evaluating the service needs of a prospective resident or current resident experiencing a change in condition, there are several resources that may be used. The most effective resources are:

- Merrill Gardens' Medical Health Statement, a history and physical, and/or a state specific form.
- Communication with discharge planners, social workers, physicians, home health providers, and/or anyone who has been involved in the recent care of this person.
- Information from prospect/resident and/or responsible party.
- Prospect's/Resident's chart at a hospital, rehabilitation center, etc.
- Progress notes (nursing, physical therapy, occupational therapy, speech therapy, home health, etc.) to assist in determining functional and health status elements of the person's condition.
- Personal observations made by Community team members (i.e., touring, dining, housekeeping).
- Family or significant others.
- Environmental observation outside the community (home, rehab, etc.) as necessary to gain additional insight into the person's abilities and needs.

Evaluation

Merrill Gardens' policy is to accept and retain only residents whose functional and health conditions meet the criteria set forth in any pertinent state regulations and are manageable within our physical and operational structure. Refer to *Criteria for Accepting and Retaining Residents*.

Whether a prospect/resident is appropriate for a Merrill Gardens community is determined through an evaluation by a Licensed Nurse or designee in conjunction with information provided by the resident's primary health care provider.

AL: *A Registered Nurse (RN) will complete a Nursing Assessment and Care Plan at move-in, monthly, post hospitalization/rehabilitation and at any change in condition for residents of the Garden House. See Alabama RN Assessments.*

AZ: *The prospect/resident will have their physician complete the Arizona Medical Health Statement, certifying that they meet the requirements of residency. The General Manager will determine if the prospect/resident is appropriate for residency at the Community based on information from both the Evaluation and the Medical Health Statement. If the individual is not yet a resident of the Community, and the Evaluation and Medical Health Statement indicate that the prospect is not appropriate for the Community, then admission will be denied. If the individual is a current resident, then a Community team member will notify the resident and/or responsible party. The Community will follow Section V of the Residency Agreement and R9-10-807 H-J.*

CA: Evaluation Interview (Pre-Placement Appraisal)

The Licensed Nurse or designee conducts the Pre-Placement Appraisal interview with the potential resident and/or responsible party to determine the level of independence and assisted living care the potential resident wants and needs. Based on the Pre-placement Appraisal and the evaluation using EHR software, the Licensed Nurse or designee will develop a service plan within five (5) days of move-in.

A licensed physician or nurse practitioner will complete the Physician's Report (LIC 602A) and, where applicable, a Tuberculosis (TB) test prior to move-in. Tuberculosis tests must not be older than twelve (12) months prior to date of move-in.

SC: *The resident will receive the initial step of the two-step tuberculin test within twenty-four (24) hours of admission to the community. The second step of the two-step tuberculin skin test must be administered within the next seven to fourteen (7 to 14) days.*

WA: *Evaluations must be performed by a Qualified Assessor within five days prior to or within fourteen (14) days a move-in. The service plan is to be completed within the first thirty (30) days of residency.*

While a preliminary screening interview may be conducted by phone, a formal evaluation interview should be conducted in person prior to move-in. The Licensed Nurse or designee will evaluate the resident using EHR software and will include the following:

- Health information that includes diagnoses and allergies
- Activities of daily living (ADL) evaluation
- Cognitive evaluation (i.e., MMSE)
- Self-medication evaluation

- Cooking evaluation (not applicable for Garden House)

In addition, a current medication list will be requested.

The General Manager or their designee makes the final determination as to whether a prospective or current resident is appropriate for admission and/or retention. If there is uncertainty about the appropriateness of a resident's functional or health status, the General Manager or their designee should contact the Vice President of Operations for their region to discuss acceptance or retention.

GA: *The community must complete an evaluation of the resident that addresses the care needs, taking into account the resident's family support, the resident's functional capacity relative to the activities of daily living, physical care needs, medical information provided, cognitive and behavioral impairments, if any, and personal preferences relative to care needs.*

WA: *If the resident is Independent and approved for residency in a 'non-licensed' apartment, the Licensed Nurse or Qualified Assessor will communicate the status of the resident to the General Manager or their designee who will complete the appropriate Residency Agreement and required move-in documentation.*

If the resident is appropriate for Assisted Living, the Licensed Nurse or Qualified Assessor will complete the evaluation and service plan using EHR software.

Service Plan

If it is determined that a prospective or current resident has care needs, the licensed nurse or designee will prepare a Service Plan. The design of the service plan will flow from the evaluation and will include the following:

- Health information that includes diagnoses and allergies.
- Information detailing the type(s) of assistance the resident will need. Resident preferences will be incorporated as much as reasonably possible.
- Frequency of service(s) that will be provided (i.e., daily, three times per week, etc.) and who will provide the service(s) (caregiver, nurse, other team member, etc.).
- Details of any service(s) provided by an outside agency or provider, including family members who may be assisting (where permitted by regulation).

WA: *Refer to Family Assistance with Medications and Treatments.*

- Level of care required for the resident based on the evaluation and identified services.
- Signature(s) of the resident and/or responsible party, and the Licensed Nurse or designee detailing the service level(s).
- The Licensed Nurse or their designee will complete the Change in Services form and provide a copy to the General Manager and Business Office Director to confirm service charges.

Where indicated by state regulations, the signatures and date on the service plan generate the required date of the next Evaluations and Service Plans for the resident. Verbal agreement will be accepted and documented in a progress note if the signature cannot be obtained prior to initiating requested services. Signature(s) should be obtained as soon as practical.

GA: *The community must document that it is providing proper skin care to prevent ulcers. If the underlying medical condition results in the resident developing a decubitus ulcer despite proper care*

being provided, the community will seek outside health care resources to assist in coordination of care and comfort measures.

SC: Using the written assessment, the Community shall develop within seven (7) days of admission a service plan with participation of the resident, administrator (or designee), and/or the sponsor or responsible party when appropriate, as evidenced by their signatures and date. The service plan shall delineate the responsibilities of the sponsor and of the Community in meeting the needs of the resident, including provisions for the sponsor to monitor the care and the effectiveness of the facility in meeting those needs. Included shall be specific goal-related objectives based on the needs of the resident as identified during the assessment phase, including adjunct support service needs, other special needs, and the methods for achieving objectives and meeting needs in measurable terms with expected achievement dates. The service plan shall be reviewed and/or revised as changes in resident needs occur, but not less than semi-annually with the resident, administrator (or designee), and/or the sponsor or responsible party as evidenced by their signatures and date.

TASK LOG

The Licensed Nurse or their designee will generate a Monthly Task Log detailing the type, method, and frequency of assistance provided by Community team members to be used/completed as follows:

- Assisted living team members will record services provided to each resident on AL services on the Task Log on a daily basis.
- A new Task Log form is printed out from the EHR software for each resident receiving AL services at the beginning of each month, or if there is a change in services.
- By the end of each shift, the caregiver initials all services that he/she assisted the resident with.
- The assigned team member should note any services not received that day on the Task Log by circling their initials in the corresponding space on the log. Additionally, the reason why the services were not provided is documented on the Task Log.
- Care that is no longer provided (discontinued) should be indicated on an updated service plan and current Task Log.

Use the Resident Task Log when:

- Documenting services provided
- Reviewing the resident's service plan
- Communicating with the responsible person and/or physician
- Reviewing the staffing needs of the Assisted Living program.

NV: All refusals of service including medical care directed by the resident's physician must be documented on the Progress Notes form along with the plan of action taken to alleviate the situation. In case of refusals of medical care directed by the resident's physician, the physician must be notified within four (4) hours after the care is rejected.

SERVICE PLAN AND TASK LOG NOTEBOOK

This record is located at the caregiver's workstation.

The current service plan should be placed in a plastic sheet protector. The Task Log and Progress Notes form should be filed behind the service plan.

At the end of the month, the Licensed Nurse or designee removes, reviews & files the completed Task Logs in each resident's chart.

Re-Evaluation and Updating the service plan

Merrill Gardens chooses to conduct a comprehensive re-Evaluation and Service Plan review on a routine basis for assisted living residents using EHR software.

The Licensed Nurse or designee will review the last six months of health and care information and update the resident's service plan to reflect changes in the resident's condition. They will notify the resident and/or responsible party that the Service Plan has been reviewed. If service level changes are needed, a team member will schedule a care conference with the resident and/or responsible party. A new Service Plan will be developed with the resident and/or responsible party, and community team member(s). Any new Service Plan should include all elements detailed previously under the Service Plan section of this policy.

AL: *A Registered Nurse (RN) will complete a Nursing Assessment and Care Plan at move-in, monthly, post hospitalization/rehabilitation and at any change in condition for residents of the Garden House. See Alabama RN Assessments.*

AZ, GA: *The Evaluation and Service Plan are reviewed every three (3) months for residents of the Garden House.*

SC: *A physical examination by the physician is to be completed annually.*

CHANGE OF CONDITION / TEMPORARY SERVICE PLAN

If the resident experiences a change in condition and it is anticipated that it will be resolved in fourteen (14) days or less, the Licensed nurse or designee will generate a temporary Service Plan using EHR software. The plan will be reviewed for effectiveness at the end of the designated period. If it is determined that the change of condition is permanent, the Licensed Nurse or designee will update the Evaluation and Service Plan and meet with the resident and/or responsible party to review the Service Plan and adjust service(s) and charges as needed.

RETURN TO COMMUNITY SERVICE PLAN

When the resident is out of the community to another health setting for more than seventy-two (72) hours, the Licensed Nurse or designee must evaluate whether the resident is appropriate to return to the community. If it is determined that the resident will return to the community, they will obtain any additional information necessary to update the Service Plan.

The Licensed Nurse or Qualified Assessor will update the Service Plan and meet with the resident and/or responsible party to review the Service Plan and adjust service(s) and charges as needed.

Charges and Fees

Charges for assisted living services are determined by the resident's Evaluation and Service Plan and are based on the service level provided. The General Manager working together with the Licensed Nurse or designee, is responsible for ensuring that residents are receiving the proper services based on their needs, and that the charges are appropriate.

Any adjustments to the Evaluation and Service Plan that increase or decrease services will be updated, and a Change of Services form should be forwarded to the General Manager and the Business Office Director.



Medical Health Statement

Merrill Gardens takes pride in providing quality assisted living services to our residents. Our license requires a health examination by a licensed physician or nurse practitioner to assist in determining whether the person is appropriate for care in our non-medical community. Your evaluation is important because we do not provide skilled nursing care. Health exams are only valid if done within 60 days prior to a resident moving into the community (30 days in Alabama & South Carolina). Please complete this form and, where applicable, perform a Tuberculosis (TB) test. The following resident/patient authorizes release of medical information to Merrill Gardens.

Resident/Patient Name: _____ Birth date: _____

Exam Date: _____ How long has Patient been with you?: _____

Tb Test or Chest X-Ray Required. Date Done: _____ Results: _____

Nevada: Annual Tb symptom check after positive test: ☐ Asymptomatic ☐ Symptomatic

Alabama: Annual Flu Vaccine Required. Date Done: _____

Alabama: Pneumonia Vaccine Required: Date Done: _____

South Carolina: See Attached Tb testing record.

If you have questions, call the General Manager or Resident Care Director at (____) _____

Physical Examination

Please (✓) if normal. Describe variations from normal, if appropriate.

Height:	Scalp:	Breasts:	Skin:
Weight:	Eyes:	Heart:	Orientation:
B/P:	Nose:	Lungs:	Appetite:
Pulse:	Mouth:	Abdomen:	Ambulation:
Respirations:	Ears:	Arms & Hands:	Sleep:
Temperature:	Neck:	Legs & Feet:	

General Health/Diagnosis

Please indicate if there are concerns or problems in any of the following areas.

Allergies (specify on next page):	Behaviors:	Disoriented/Confused:	Drug or Alcohol Abuse (circle):
Falling:	Incontinence:	Infection (specify):	Other:

Current Diagnoses:

Significant Historical Diagnoses:

Overall Health and Plan of Care

Ability to manage medications: ☐ Self-Administer ☐ Assist/Supervise ☐ Administer by Nurse

☐ Current Medication(s), and for what condition(s)? ☐ May Refill

Please attach prescriptions for all medications and any over the counter products such as vitamins, etc.

Special medical care requiring treatment outside Retirement Community?

☐ Chemotherapy ☐ Radiation
☐ Dialysis ☐ Other

History and Current Health Status:

☐ History of Mental Illness
☐ Significant Surgical History
☐ Physical Impairments

Ambulatory Status: ☐ Independent ☐ Cane ☐ Walker ☐ Wheelchair ☐ Non Ambulatory

☐ Allergies

Capacity for Self-Care: Any limitations? ☐ Yes ☐ No

Does this person need assistance with (check for yes)

☐ grooming? ☐ bathing? ☐ dressing? ☐ toileting?

☐ Yes ☐ No Is this person free of contagious disease?

☐ Yes ☐ No Does this person need continuous supervision?

☐ Yes ☐ No Does this person require chemical/physical restraints?

☐ Yes ☐ No Has this person been assessed for depression?

☐ Yes ☐ No Has this person been checked for decubitus?

☐ Yes ☐ No If this person has some form of **dementia**, do they require a secure unit for their own safety?

Nevada: Capacity for Self-Preservation: Ability to evacuate from sleeping area to past a smoke barrier within 4 minutes. ☐ Independent ☐ Requires staff assistance

South Carolina: Requires continuous daily supervision of a licensed nurse: ☐ Yes ☐ No

Appropriate for placement in a Community Residential Care Facility: ☐ Yes ☐ No

Overall Health: ☐ Good ☐ Fair ☐ Poor **Comments:**

Please return this form to Merrill Gardens in the envelope provided

Print Name: Physician/Nurse Practitioner Date

Signature: Physician/Nurse Practitioner

Address

Section 1-2a 10/12/18

()

Phone



AZ Medical Health Statement

Merrill Gardens takes pride in providing quality assisted living services to our residents. Our license requires a health examination by a licensed physician or nurse practitioner to assist in determining whether the person is appropriate for care in our non-medical community. Your evaluation is important because we do not provide skilled nursing care. Health exams are only valid if done within 60 days prior to a resident moving into the community. Please complete this form and, where applicable, perform a Tuberculosis (TB) test. The following resident/patient authorizes release of medical information to Merrill Gardens.

Resident/Patient Name: _____ Birth date: _____

Exam Date: _____ How long has Patient been with you? _____

Tb Test or Chest X-Ray Required. Date Done: _____ Results: _____ mm

Tb symptom check: ☐ Asymptomatic ☐ Symptomatic

If you have questions, call the General Manager or Resident Care Director at (____) _____

Physical Examination

Please (✓) if normal. Describe variations from normal, if appropriate.

Height:	Scalp:	Breasts:	Skin:
Weight:	Eyes:	Heart:	Orientation:
B/P:	Nose:	Lungs:	Appetite:
Pulse:	Mouth:	Abdomen:	Ambulation:
Respirations:	Ears:	Arms & Hands:	Sleep:
Temperature:	Neck:	Legs & Feet:	

General Health/Diagnosis

Please indicate if there are concerns or problems in any of the following areas.

Allergies (specify on next page):	Behaviors:	Disoriented/Confused:	Drug or Alcohol Abuse (circle):
Falling:	Incontinence:	Infection (specify):	Other:

Current Diagnoses:

Significant Historical Diagnoses:

Overall Health and Plan of Care

Requires Assisted Living level of care ☐ Yes ☐ No

Requires Continuous medical services (acute care hospital level of care) ☐ Yes ☐ No

Requires Continuous Nursing services (skilled nursing facility level of care) ☐ Yes ☐ No

Requires Intermittent Nursing services (home health care or hospice) ☐ Yes ☐ No

Requires Continuous Behavioral Health (acute care psychiatric) ☐ Yes ☐ No

Requires Restraints ☐ Yes ☐ No

Ability to manage medications: ☐ No Assistance ☐ Assist with Self-Administration ☐ Administration

☐ Trained Certified Caregivers have permission to administer medications.

☐ Flu Vaccine Annually ☐ Mantoux skin test or other diagnostic study Annually to rule out Tb

☐ Pneumonia Vaccine when needed ☐ May have alcoholic beverages

☐ Current Medication(s), and for what condition(s)? ☐ May Refill

Please attach prescriptions for all medications and any over the counter products such as vitamins, etc.

Special medical care requiring treatment outside Retirement Community?

☐ Chemotherapy _____ ☐ Radiation _____
☐ Dialysis _____ ☐ Other _____

History and Current Health Status:

☐ History of Mental Illness _____

☐ Significant Surgical History _____

☐ Physical Impairments _____

Ambulatory Status: ☐ Independent ☐ Cane ☐ Walker ☐ Wheelchair ☐ Non Ambulatory

☐ Allergies _____

Capacity for Self-Care: Any limitations? ☐ Yes ☐ No _____

Does this person need assistance with (check for **yes**)

☐ grooming? _____ ☐ bathing? _____ ☐ dressing? _____ ☐ toileting? _____

☐ Yes ☐ No Is this person free of contagious disease?

☐ Yes ☐ No Does this person need continuous supervision?

☐ Yes ☐ No Does this person require chemical/physical restraints?

☐ Yes ☐ No Has this person been assessed for depression?

☐ Yes ☐ No Has this person been checked for decubitus?

☐ Yes ☐ No If this person has some form of **dementia**, do they require a secure unit for their own safety?

Overall Health: ☐ Good ☐ Fair ☐ Poor Comments: _____

Please return this form to Merrill Gardens in the envelope provided

Print Name: Physician/Nurse Practitioner

Date

Signature: Physician/Nurse Practitioner

Address

(____) _____
Phone

Resident Name:

1. *Chlorophyll a* (Chl a) is the primary photosynthetic pigment in most plants and algae. It is a green pigment that absorbs light energy in the blue and red regions of the visible spectrum. Chl a is essential for the light-dependent reactions of photosynthesis, where it converts light energy into chemical energy in the form of ATP and NADPH.

AL Care Level 1: 1-125 pts, Add'l Levels are 100 pts (i.e. Care Level 2: 126-225 pts)

GH Care Level 1: 1-425 pts, Add'l Levels are 100 pts (i.e. Care Level 2: 426-525 pts)

10/9/11

Notes:-

Notes:

Independent Living Evaluation - WA/CA/AL

Resident Name: _____ Gender: ☐ Female ☐ Male
Date of Birth: _____ Care Level: _____ Unit: _____
Assessment Date: _____ Reason for Assessment: _____
Comments: _____

Section: IL - Mobility/Ambulation

Instructions: FOR RESIDENTS WHO REQUIRE ASSISTANCE FROM AN OUTSIDE SERVICE PROVIDER OR MG STAFF, PLEASE DISCONTINUE THE INDEPENDENT LIVING EVALUATION AND PERFORM MG HSE.

- * **Mobility/Ambulation** (Select one)
- ☐ Independent: Resident does not require assistance with mobility/ambulation ~ document mobility devices in note section.
 - ☐ Resident requires assistance with mobility/ambulation from an outside service provider.
 - ☐ Resident requires staff assistance with mobility/ambulation.

Notes: _____

Section: IL - Medications

Instructions: FOR RESIDENTS WHO REQUIRE ASSISTANCE FROM AN OUTSIDE SERVICE PROVIDER OR MG STAFF, PLEASE DISCONTINUE THE INDEPENDENT LIVING EVALUATION AND PERFORM MG HSE.

- * **Medications** (Select one)
- ☐ Independent: Resident does not require assistance with medication administration and is able to complete self-medication evaluation.
 - ☐ Resident requires assistance with medications from an outside provider.
 - ☐ Resident requires staff assistance with medications and is unable able to pass self-medication evaluation.

Notes: _____

Section: Self-Administration of Medication Evaluation

Instructions: Complete upon initial evaluation and review during completion of re-evaluation/change of condition.

- * **Does the resident currently self-administer his or her medication?** (Select one)
- ☐ Yes
 - ☐ No
 - ☐ Sometimes

Resident Name: _____

Independent Living Evaluation - WA/CA/AL

* Can the resident state what medication they take (i.e., prescribed, over-the-counter, supplements, etc.)? The resident may use a list or read off of packaging when responding to this question. (Select one)

- ☐ Yes
☐ No
☐ Sometimes

* Can the resident state what the medication is used for? (Select one)

- ☐ Yes
☐ No
☐ Sometimes

* Can the resident state when they take each medication? (Select one)

- ☐ Yes
☐ No
☐ Sometimes

* Can the resident open a container? (Select one)

- ☐ Yes
☐ No

* Can the resident read the medication label? (Select one)

- ☐ Yes
☐ No

* Can the resident identify the proper medication dosage? (Select one)

- ☐ Yes
☐ No
☐ Sometimes

* Is there a reported history of non-compliance with a medication regimen? (Select one)

- ☐ Yes
☐ No

* Can the resident report adverse affects of medication? (Select one)

- ☐ Yes
☐ No
☐ Sometimes

* Physician's order obtained for self-administration of medication? Note that this is not required in the state of Washington. (Select one)

- ☐ Yes
☐ No
☐ N/A

Resident Name: _____

Independent Living Evaluation - WA/CA/AL

- * If family is assisting, has the Family Assistance form been completed? (Select one)
This form is only required in WA state.

- ☐ Yes
☐ No
☐ N/A

Section: Self-Administration of Medication Evaluation Score

- * Total Score: Self-Administration of Medication Evaluation (Select one)

- ☐ 0 - 4 Resident demonstrates ability to self-administer medications.
☐ 5 - 15 Resident does not demonstrate the ability to self-administer medications and requires assistance.

Section: IL - Bathing/Showers

Instructions: FOR RESIDENTS WHO REQUIRE ASSISTANCE FROM AN OUTSIDE SERVICE PROVIDER OR MG STAFF, PLEASE DISCONTINUE THE INDEPENDENT LIVING EVALUATION AND PERFORM MG HSE.

- * Bathing/Showers (Select one)

- ☐ Independent: Resident does not require assistance with bathing.
☐ Resident requires assistance with bathing from an outside provider.
☐ Resident requires staff assistance with bathing.

Notes: _____

Section: IL - Grooming/Personal Hygiene

Instructions: FOR RESIDENTS WHO REQUIRE ASSISTANCE FROM AN OUTSIDE SERVICE PROVIDER OR MG STAFF, PLEASE DISCONTINUE THE INDEPENDENT LIVING EVALUATION AND PERFORM MG HSE.

- * Grooming/Personal Hygiene (Select one)

- ☐ Independent: Resident does not require assistance with grooming/personal hygiene.
☐ Resident requires assistance with grooming/personal hygiene from an outside provider.
☐ Resident requires staff assistance with grooming/personal hygiene.

Notes: _____

Section: IL - Incontinence

Instructions: FOR RESIDENTS WHO REQUIRE ASSISTANCE FROM AN OUTSIDE SERVICE PROVIDER OR MG STAFF, PLEASE DISCONTINUE THE INDEPENDENT LIVING EVALUATION AND PERFORM MG HSE.

- * Incontinence (Select one)

- ☐ Not applicable – resident has no reported incontinence.
☐ Independent: Resident self-manages incontinence.
☐ Resident is incontinent and requires assistance from an outside provider to manage hygiene and/or supplies.
☐ Resident is incontinent and requires staff assistance to manage.

Notes: _____

Resident Name: _____

Independent Living Evaluation - WA/CA/AL

Section: IL - Dressing

Instructions: FOR RESIDENTS WHO REQUIRE ASSISTANCE FROM AN OUTSIDE SERVICE PROVIDER OR MG STAFF, PLEASE DISCONTINUE THE INDEPENDENT LIVING EVALUATION AND PERFORM MG HSE.

* Dressing (Select one)

- ☐ Independent: Resident does not require assistance with dressing.
- ☐ Resident requires assistance with dressing from an outside provider.
- ☐ Resident requires staff assistance with dressing.

Notes: _____

Section: IL - Meal Consumption

Instructions: FOR RESIDENTS WHO REQUIRE ASSISTANCE FROM AN OUTSIDE SERVICE PROVIDER OR MG STAFF, PLEASE DISCONTINUE THE INDEPENDENT LIVING EVALUATION AND PERFORM MG HSE.

* Meal Consumption (Select one)

- ☐ Independent: Resident does not require assistance with meal consumption.
- ☐ Resident does require assistance with meal consumption from an outside service provider.
- ☐ Resident requires staff assistance with meal consumption.

Notes: _____

Section: IL - Decision Making

Instructions: FOR RESIDENTS WHO REQUIRE ASSISTANCE FROM AN OUTSIDE SERVICE PROVIDER OR MG STAFF, PLEASE DISCONTINUE THE INDEPENDENT LIVING EVALUATION AND PERFORM MG HSE.

* Decision Making (Select one)

- ☐ No impairment: Resident recognizes and makes own decisions.
- ☐ Resident not able to recognize when to make a decision, but follows directions.
- ☐ Resident unable to recognize when to make decisions and/or cannot follow direction, requires staff assistance.

Notes: _____

Section: IL - Orientation

Instructions: FOR RESIDENTS WHO REQUIRE ASSISTANCE FROM AN OUTSIDE SERVICE PROVIDER OR MG STAFF, PLEASE DISCONTINUE THE INDEPENDENT LIVING EVALUATION AND PERFORM MG HSE.

* Orientation (Select one)

- ☐ No impairment: Oriented to person, place, time and situation.
- ☐ Resident has difficulty with orientation to person, place, time or situation.
- ☐ Resident has current or history of frequent disorientation to person, place, time or situation and requires supervision and/or oversight from staff for safety.

Notes: _____

Resident Name: _____

Independent Living Evaluation - WA/CA/AL

Section: IL - Behavior

Instructions: FOR RESIDENTS WHO REQUIRE ASSISTANCE FROM AN OUTSIDE SERVICE PROVIDER OR MG STAFF, PLEASE DISCONTINUE THE INDEPENDENT LIVING EVALUATION AND PERFORM MG HSE.

*** Behavior** (Select one)

- ☐ No impairment: Resident does not have current or history of disruptive, aggressive, verbal or socially inappropriate behaviors.
- ☐ Resident has demonstrated 1 or more behaviors above (1 time a week or less).
- ☐ Resident has demonstrated 1 or more behaviors above (more than 1 time a week). Requires supervision and/or oversight from staff.

Notes: _____

Section: IL - Smoking

Instructions: FOR RESIDENTS WHO REQUIRE ASSISTANCE FROM AN OUTSIDE SERVICE PROVIDER OR MG STAFF, PLEASE DISCONTINUE THE INDEPENDENT LIVING EVALUATION AND PERFORM MG HSE.

*** Smoking** (Select one)

- ☐ Resident does not currently smoke
- ☐ Independent: Resident currently smokes and understands MG Policy and Procedure.
- ☐ Resident requires staff assistance with smoking.

Notes: _____

Section: IL - Oxygen Use

Instructions: FOR RESIDENTS WHO REQUIRE ASSISTANCE FROM AN OUTSIDE SERVICE PROVIDER OR MG STAFF, PLEASE DISCONTINUE THE INDEPENDENT LIVING EVALUATION AND PERFORM MG HSE.

*** Oxygen Use** (Select all that apply)

- ☐ Independent: Resident uses supplemental oxygen and is independent with all aspects of oxygen utilization.
- ☐ Resident requires staff assistance with supplemental oxygen.
- ☐ Resident understands MG Policy and Procedure on signage, appropriate storage and usage of all equipment.

Notes: _____

Section: IL - Overall Skin Condition

Instructions: FOR RESIDENTS WHO REQUIRE ASSISTANCE FROM AN OUTSIDE SERVICE PROVIDER OR MG STAFF, PLEASE DISCONTINUE THE INDEPENDENT LIVING EVALUATION AND PERFORM MG HSE.

*** Overall Skin Condition** (Select one)

- ☐ Resident denies any skin integrity issues.
- ☐ Resident has altered skin integrity and/or wound and is able to self-manage – (Document in the note section).
- ☐ Resident has altered skin integrity and/or wound and requires assistance from an outside service provider.
- ☐ Resident has altered skin integrity and/or wound and requires staff assistance.

Notes: _____

Resident Name: _____

Independent Living Evaluation - WA/CA/AL

Section: IL - Emergency Response

Instructions: FOR RESIDENTS WHO REQUIRE ASSISTANCE FROM AN OUTSIDE SERVICE PROVIDER OR MG STAFF, PLEASE DISCONTINUE THE INDEPENDENT LIVING EVALUATION AND PERFORM MG HSE.

* **Emergency Response** (Select all that apply)

- ☐ Resident participates in daily check-in.
- ☐ Emergency Pull Cord
- ☐ Pendant Call System
- ☐ Other - Resident has enabling device(s) used for emergency response - (Document in note section).

Notes: _____

Section: IL - Outside Service Provider

Instructions: FOR RESIDENTS WHO REQUIRE ASSISTANCE FROM AN OUTSIDE SERVICE PROVIDER OR MG STAFF, PLEASE DISCONTINUE THE INDEPENDENT LIVING EVALUATION AND PERFORM MG HSE.

* **Outside Service Provider** (Select one).

- ☐ Not Applicable - Resident does not receive service from any outside service providers (Including but not limited to: PT, OT, ST, Social Worker, Hospice, Palliative Care, RN, LPN, LVN, Home Health Aide or Private Duty Caregiver)
- ☐ Resident has hired outside service provider and is independent in managing all aspects (Please document information in notes and perform MG HSE).
- ☐ Outside Service Provider understands MG Policy and Procedure and Acknowledgement & Indemnification has been signed

Notes: _____

Section: Cooking Evaluation

Instructions: Complete upon initial evaluation and review during completion of re-evaluation/change of condition.

* **Can resident turn stove burners on and off?** (Select one)

- ☐ Yes
- ☐ No

* **Does resident check oven for contents prior to turning on?** (Select one)

- ☐ Yes
- ☐ No

* **Can resident turn oven on and off?** (Select one)

- ☐ Yes
- ☐ No

* **Can resident demonstrate use of microwave by heating a cup of water?** (Select one)

- ☐ Yes
- ☐ No

* **Does resident use appliances appropriately?** (Select one)

- ☐ Yes
- ☐ No

Resident Name: _____

Independent Living Evaluation - WA/CA/AL

* Does resident have a diagnosis of Cognitive Impairment? (Select one)

- ☐ Yes
☐ No

* Based upon staff evaluation, is the resident safe to cook unattended? (Select one)

- ☐ Yes
☐ No

* If no, has the stove/oven breaker been turned off? (Select one)

- ☐ Yes
☐ No
☐ N/A

Section: IL - Evaluation Summary

* Summary (Select one)

- ☐ RESIDENT IS INDEPENDENT - Select this response if, based on the questions above, the resident has been evaluated as Independent and needs no assisted living services at this time.
- ☐ RESIDENT IS NOT INDEPENDENT - Select this response if, based on the questions above, the resident has been evaluated and needs or receives assisted living services from MG or an outside service provider.

Notes: _____

Signatures:

Resident _____ Date: _____

Community Representative _____ Date: _____

Responsible Party _____ Date: _____

Community Representative _____ Date: _____

MG HSE WA

Resident Name: _____ Gender: ☐ Female ☐ Male
Date of Birth: _____ Care Level: _____ Unit: _____
Assessment Date: _____ Reason for Assessment: _____
Comments: _____

Section: Demographics

Admitted From (Select one)

- ☐ Private Home/Apt.
- ☐ Private Home/Apt. with supportive services
- ☐ Private Home/Apt. with family
- ☐ Other board and care/assisted living/group home
- ☐ Nursing home
- ☐ Acute care hospital
- ☐ Psychiatric hospital
- ☐ MR/DD facility
- ☐ Rehabilitation hospital
- ☐ Other (describe in Notes below)

Notes: _____

Move In Date _____

* Gender _____

Date of Birth _____

Place of Birth _____

Marital Status _____

Religion _____

Spiritual
Preferences _____

Education (Highest Level Completed) (Select one)

- ☐ No schooling
- ☐ 8th grade or less
- ☐ 9-11 grades
- ☐ High school
- ☐ Technical/trade school
- ☐ Some college
- ☐ Bachelor's degree
- ☐ Graduate degree

Resident Name: _____

MG HSE WA

Lifetime
Occupation _____

Primary Spoken
Language _____

Primary Written
Language _____

Responsibility/Legal Guardian (Select all that apply)

- ☐ Legal guardian
- ☐ Conservator
- ☐ Other Legal Oversight
- ☐ Durable power of attorney/health care
- ☐ Durable power of attorney/financial
- ☐ Family member responsible
- ☐ Resident responsible for self
- ☐ None Apply

Section: Diagnoses

* Diagnoses _____

* Diagnoses - Reviewed (Select one)

- ☐ The resident's active diagnoses were reviewed. See Service Plan for details.

* Allergies _____

* Allergies - Reviewed (Select one)

- ☐ The resident's active allergies were reviewed. See Service Plan for details.

* Communicable Diseases (Select one)

- ☐ Resident is free from communicable disease(s)
- ☐ Resident is not free from communicable disease(s)

* Smoking (Select one)

- ☐ Resident does not currently smoke
- ☐ Resident currently smokes
- ☐ Resident smokes, requires staff supervision with the smoking

* Oxygen Use (Select one)

☐ In House ☐ Outside Service ☐ Shared Responsibility

- ☐ Not Applicable: Resident does not require supplemental oxygen
- ☐ Independent: Resident requires supplemental oxygen and is independent with all aspects of oxygen utilization
- ☐ Minimal: Resident requires cueing/reminders to use supplemental oxygen as ordered by MD
- ☐ Moderate: Resident requires some hands on assistance with oxygen such as adjusting nasal cannula, changing tubing, ordering supplies
- ☐ Extensive: Resident requires assistance from a licensed nurse to adjust liters per minute, and/or monitoring, etc.

Resident Name: _____

MG HSE WA

*** Oxygen Enabling Devices and Methods** (Select all that apply)

- ☐ Nasal cannula
- ☐ Non-Rebreather mask
- ☐ Portable oxygen tank
- ☐ Oxygen concentrator
- ☐ CPAP/BIPAP
- ☐ Other - Resident has enabling device(s) used related to oxygenation (see Service Plan)
- ☐ None Apply

Section: Vitals / Vitals Monitoring

Height _____

Weight _____

Blood Pressure _____

Pulse _____

Respiration _____

Temperature _____

*** Vital Monitoring (Not applicable in NV)** (Select all that apply)

- ☐ Weights (Daily)
- ☐ Weights (2x Monthly)
- ☐ Weights (Weekly)
- ☐ Blood Pressure (Daily)
- ☐ Blood Pressure (approx every other day)
- ☐ Blood Pressure (Weekly)
- ☐ Pulse (Daily)
- ☐ Pulse (Weekly)
- ☐ Temperature (Daily)
- ☐ Temperature (Weekly)
- ☐ Respiration (Daily)
- ☐ Respiration (Weekly)

Section: Neurocognitive

*** Orientation Evaluation** (Select all that apply)

- ☐ Resident is oriented to person
- ☐ Resident is oriented to place
- ☐ Resident is oriented to time
- ☐ Resident is oriented to situation
- ☐ Short term memory deficit
- ☐ Long term memory deficit
- ☐ None Apply

Resident Name: _____

MG HSE WA

* **Orientation** (Select one)

☐ In House ☐ Outside Service ☐ Shared Responsibility

- ☐ No Assistance: Oriented to person, place, time and situation
- ☐ Mild Assistance: Resident has current or history of occasional disorientation to person, place, time or situation that does not interfere with functioning in familiar surroundings. Requires some direction and reminding from others
- ☐ Moderate Assistance: Resident has current or history of occasional disorientation to person, place, time or situation even in familiar surroundings and requires supervision and oversight for safety
- ☐ Severe Assistance: Resident is frequently disoriented and may require repeated verbal prompts and/or direction

* **Orientation Additional Services** (Select one)

- ☐ Yes, Resident requires additional services with orientation.
- ☐ No

* **Memory Deficit** (Select one)

☐ In House ☐ Outside Service ☐ Shared Responsibility

- ☐ No Assistance: Resident does not have memory deficit
- ☐ Mild Assistance: Resident has current or history of occasional difficulty remembering and using information; requires some directions and reminding from others; may have difficulty following written instructions
- ☐ Moderate Assistance: Current or history of frequent or difficulty remembering and using information; requires directions and reminding from others; cannot follow written instructions
- ☐ Severe Assistance: Resident is unable to remember or use information; may require repeated verbal prompts and/or direction

* **Memory Deficit: Additional Services** (Select one)

- ☐ Yes, Resident requires additional services with memory deficit.
- ☐ No

Other Sensory Impairment(s) (Select all that apply)

- ☐ Resident has an Olfactory impairment (sense of smell)
- ☐ Resident has a Tactile impairment (sense of touch)
- ☐ Resident has a Gustatory impairment (sense of taste)
- ☐ None Apply

* **Communication** (Select one)

- ☐ No Assistance: Resident is able to communicate effectively and make needs known, with or without assistive device(s)
- ☐ Mild Assistance: Resident has current or history of occasional difficulty communicating and receiving information; may have occasional difficulty following instructions with using communication devices.
- ☐ Moderate Assistance: Resident has current or history of frequent difficulty communicating and receiving information; has difficulty following instructions; has frequent difficulty following instructions using communication devices.
- ☐ Severe Assistance: Resident is unable to communicate or receive information; is unable to follow instructions with using communication devices.

Resident Name: _____

MG HSE WA

*** Communication Enabling Devices and Methods** (Select all that apply)

- ☐ Resident uses a writing board
- ☐ Resident uses an electronic device such as a tablet or computer
- ☐ Resident uses gestures
- ☐ Other - Resident has enabling device(s) used for communication device (see Service Plan)
- ☐ None Apply

Section: Psychosocial

*** Wandering** (Select one)

- ☐ Resident does not have current or history of wandering.
- ☐ Resident has current or history of wandering within the residence or facility and may wander outside, but does not jeopardize health or safety (of self or others).
- ☐ Resident has current or history of wandering within the residence of the facility. May wander outside; health or safety may be jeopardized, but participant is not combative about returning.
- ☐ Resident wanders outside and leaves immediate area. Has history of leaving immediate area, getting lost, or being combative about returning. Requires supervision.

*** Behaviors** (Select one)

- ☐ Resident does not have current or history of disruptive, aggressive, anxious, depressive, verbal or socially inappropriate behaviors.
- ☐ Resident has current or history of minimal disruptive, aggressive, anxious, depressive, or socially inappropriate behavior, either verbally or physically improper.
- ☐ Resident has current or history of moderate disruptive, aggressive, anxious, depressive, or socially inappropriate behavior, either verbally or physically improper.
- ☐ Resident is verbally or physically inappropriate and requires supervision.

*** Behaviors - Additional Services** (Select one)

- ☐ Yes, Resident requires additional services with behaviors.
- ☐ No

*** Hallucinations/Delusions** (Select one)

- ☐ Resident does not have current or history of hallucinations/delusions
- ☐ Resident has current or history of occasional hallucinations/delusions
- ☐ Resident has current or history of frequent hallucinations/delusions
- ☐ Resident has current or history of chronic hallucinations/delusions

*** Harm to Self, Others and/or Property** (Select one)

- ☐ Resident does not have current or history of harming self, others and/or property
- ☐ Resident has current or history of occasional harming self, others and/or property
- ☐ Resident has current or history of frequent harming self, others and/or property
- ☐ Resident has current or history of chronic harming self, others and/or property

Resident Name: _____

MG HSE WA

*** Substance Use**

(Select one)

☐ In House ☐ Outside Service ☐ Shared Responsibility

- ☐ Resident does not currently or have a history of substance use.
- ☐ Resident has current or history of substance use which may cause some interpersonal and/or health problems, but does not significantly impair overall independent functioning.
- ☐ Resident has current or history of substance use which may cause moderate problems with peers, family members, law officials, etc.
- ☐ Resident has current or history of frequent substance use which causes significant problems with others and severely impairs ability to function independently.

Involvement of Resident's Family

(Select one)

- ☐ Family is involved with physical and/or emotional needs
- ☐ Family is cooperative but cannot provide ongoing physical and/or emotional support
- ☐ Family lives out of town, unable to visit but would like updates on resident's care needs
- ☐ Family has no involvement with resident

Involvement of Resident's Friends and Associates

(Select one)

- ☐ Friends and associates are involved with physical and/or emotional needs
- ☐ Friends and associates are cooperative but cannot provide ongoing physical and/or emotional support
- ☐ Friends and associates live out of town, unable to visit but would like updates on resident's care needs
- ☐ Friends and associates have no involvement with resident

Section: Vision & Hearing

*** Vision - Level of Impairment**

(Select one)

☐ In House ☐ Outside Service ☐ Shared Responsibility

- ☐ No Impairment: Resident does not have visual impairment
- ☐ Mild Impairment: Resident has mild visual impairment but can see adequately with devices
- ☐ Moderate Impairment: Resident has moderate visual impairment
- ☐ Severe Impairment: Resident has severe visual impairment
- ☐ Legally Blind: Resident is legally blind

*** Vision - Level of Assistance**

(Select one)

☐ In House ☐ Outside Service ☐ Shared Responsibility

- ☐ Independent: Resident can self manage vision needs. May or may not use vision devices.
- ☐ Moderate Assistance: Resident requires moderate assistance from staff for vision needs.
- ☐ Extensive Assistance: Resident requires frequent assistance from staff for vision needs.

*** Hearing - Level of Impairment**

(Select one)

☐ In House ☐ Outside Service ☐ Shared Responsibility

- ☐ No Impairment: Resident does not have hearing impairment
- ☐ Mild Impairment: Resident has mild hearing impairment but can hear adequately with devices
- ☐ Moderate Impairment: Resident has moderate hearing impairment
- ☐ Severe Impairment: Resident has severe hearing impairment
- ☐ Total Hearing Loss: Resident is unable to hear

Resident Name: _____

MG HSE WA

* **Hearing - Level of Assistance** (Select one) ☐ In House ☐ Outside Service ☐ Shared Responsibility

- ☐ Independent: Resident can self manage hearing needs. May or may not use hearing devices.
- ☐ Moderate Assistance: Resident requires moderate assistance from staff for hearing needs.
- ☐ Extensive Assistance: Resident requires frequent assistance from staff for hearing needs.

Section: Fall Potential Evaluation

Instructions: Please choose the most appropriate responses based on resident's current level of function upon initial evaluation and review during completion of re-evaluation/change of condition. Fall potential score will be displayed in the next section.

* **Falls within the last 90 days** (Select one)

- ☐ No falls
- ☐ 1-2 falls in the last 90 days
- ☐ 3 or more falls in last 90 days

* **List of predisposing diseases (including but not limited to)** (Select all that apply)

- ☐ Hypotension
- ☐ Hypertension
- ☐ Vertigo
- ☐ CVA
- ☐ Parkinson's
- ☐ Loss of limb(s)
- ☐ Seizures
- ☐ Arthritis
- ☐ Osteoporosis
- ☐ Fractures
- ☐ COPD
- ☐ CHF
- ☐ CAD
- ☐ CA
- ☐ Diabetes
- ☐ None Apply

* **Number of predisposing diseases checked off above** (Select one)

- ☐ None present
- ☐ 1-2 present
- ☐ 3 or more present

* **Gait/Balance** (Select one)

- ☐ Normal
- ☐ Uses assistive device
- ☐ Chair-bound

* **Elimination/Continence** (Select one)

- ☐ Ambulatory/Continent
- ☐ Ambulatory/Incontinent
- ☐ Chair-bound, requires assistance with ambulation

Resident Name: _____

MG HSE WA

* **List of medications (including but not limited to)** (Select all that apply)

- ☐ Anesthetics
- ☐ Anticoagulants
- ☐ Antihistamines
- ☐ Antihypertensives
- ☐ Antiseizure
- ☐ Benzodiazepines
- ☐ Cathartics
- ☐ Diuretics
- ☐ Hypoglycemics
- ☐ Narcotics
- ☐ Psychotropics
- ☐ Sedatives/hypnotics
- ☐ None Apply

* **Medications** (Select one)

- ☐ None taken in last 14 days
- ☐ Takes 1-2
- ☐ Takes 3 or more

* **Medication changes in last 5 days** (Select one)

- ☐ Yes
- ☐ No

* **Alcohol use** (Select one)

- ☐ Yes
- ☐ No

* **Environmental room safety issues (including but not limited to)** (Select all that apply)

- ☐ Throw rugs
- ☐ Improper shoes (describe if selected)
- ☐ Room clutter
- ☐ All types of cords
- ☐ Inadequate lighting
- ☐ Stairs
- ☐ Other
- ☐ None Apply

* **Environmental room safety issues** (Select one)

- ☐ None present
- ☐ 1-2 present
- ☐ 3 or more present

* **Sensory/Vision/Hearing** (Select one)

- ☐ Adequate with or without devices
- ☐ Poor with or without devices
- ☐ Legally blind/deaf

Resident Name: _____

MG HSE WA

* **Orientation** (Select one)

- ☐ No Impairment
- ☐ Mild impairment
- ☐ Moderate Impairment
- ☐ Severe impairment

Section: Fall Potential Evaluation Score

* **Total Score: Fall Potential Evaluation** (Select one)

- ☐ 0 - 1 Resident is not a potential for falls
- ☐ 2 - 5 Resident is a low potential for falls
- ☐ 6 - 9 Resident is a moderate potential for falls
- ☐ 10 - 26 Resident is a high potential for falls

Section: Mobility/Ambulation/Escorting

* **Mobility/Ambulation/Escorting – Level of Assistance** (Select one)

- ☐ Resident does not require staff assistance with mobility/ambulation or escorting
- ☐ Minimal: Resident may require verbal reminders/cueing to and from meals, activities, and/or common areas with or without devices. No hands on assistance
- ☐ Moderate: Resident may require prompts/cues for mobility/ambulation or escorting and or reminders/encouragement to utilize assistive devices to and from meals, activities and/or common areas and may require some hands on assistance
- ☐ Extensive: Resident may require escorts with or without the use of assistive devices to and from meals, activities and/or common areas
- ☐ Total: Resident requires hands on assistance with escorting with or without devices to and from meals, activities and/or common areas

* **Mobility** (Select one)

- ☐ Normal
- ☐ Decreased

* **Balance** (Select one)

- ☐ Normal
- ☐ Decreased

* **Paralysis** (Select all that apply)

- ☐ Left Side
- ☐ Right Side
- ☐ Lower Body
- ☐ Upper Body
- ☐ General
- ☐ None Apply

* **Amputation** (Select one)

- ☐ Yes
- ☐ No

Resident Name: _____

MG HSE WA

* Gait (Select one)

- ☐ Normal
☐ Decreased

Resident Is Able To (Select all that apply)

- ☐ Walk
☐ Wheel
☐ Climb Stairs
☐ None Apply

* Mobility/Ambulation Enabling Devices and Methods (Select all that apply)

- ☐ Brace/orthotic appliance
☐ Cane (regular, quad)
☐ Crutch
☐ Prosthesis
☐ Scooter/Electric Wheelchair/Motorized Device
☐ Walker (no wheels, wheeled, seat attached)
☐ Wheelchair
☐ Other - Resident has other enabling device(s) used for mobility/ambulation (see Service Plan)
☐ None Apply

Section: Medical Device Evaluation: Section A

Instructions: For residents using of any fixed mobility device (transfer pole, half [1/2] siderail, bed cane, grab bar [bathroom], fixed arm rests on a toilet, shower chair, transfer bench, hospital bed): complete upon initial evaluation and review during completion of re-evaluation/change of condition. Otherwise, select not applicable. DOES NOT APPLY TO STANDARD BUILDING FEATURES (i.e. HANDRAILS IN BATHROOMS & HALLWAYS). N/A in Alabama for transfer pole, 1/2 rails, bed canes and transfer bench. N/A in Georgia for 1/2 rails.

* Medical Device (s): _____

* Is the resident non-ambulatory? (Select one)

- ☐ Yes
☐ No

* Does the resident's level of consciousness fluctuate? (Select one)

- ☐ Yes
☐ No

* Does the resident have alteration in safety awareness due to cognitive impairment? (Select one)

- ☐ Yes
☐ No

* Does the resident have a history of falls? (Select one)

- ☐ Yes
☐ No

* Has the resident displayed poor bed mobility or difficulty moving to a sitting position on the side of the bed? (Select one)

- ☐ Yes
☐ No

Resident Name: _____

MG HSE WA

* Does the resident have difficulty with balance or poor trunk control? (Select one)

- ☐ Yes
☐ No

* Does the resident have postural hypotension? (Select one)

- ☐ Yes
☐ No

* Is the resident on any medications which require safety precautions? (Select one)

- ☐ Yes
☐ No

* Is the resident currently using the device for positioning or support? (Select one)

- ☐ Yes
☐ No

* Has the resident expressed a desire to have the device used for safety and/or comfort? (Select one)

- ☐ Yes
☐ No

* Is the resident visually challenged? (Select one)

- ☐ Yes
☐ No

Section: Medical Device Evaluation: Section B

Instructions:

Complete upon initial, routine and change of condition evaluation for residents using of any fixed mobility device (transfer pole, half [1/2] siderail, bed cane, grab bar [bathroom], fixed arm rests on a toilet, shower chair, transfer bench, hospital bed). Otherwise, select not applicable. DOES NOT APPLY TO STANDARD BUILDING FEATURES (i.e. HANDRAILS IN BATHROOMS & HALLWAYS). N/A in Alabama for transfer pole, 1/2 rails, bed canes and transfer bench. N/A in Georgia for 1/2 rails.

* Does the medical device appear to be in good working order/condition? (Select one)

- ☐ Yes
☐ No

* Is the resident able to use the medical device safely? (Select one)

- ☐ Yes
☐ No

* Has the staff been trained/instructed on proper and safe use of the medical device? (Select one)

- ☐ Yes
☐ No

* Does the medical device enhance the resident's ability to maintain mobility and/or independence? (Select one)

- ☐ Yes
☐ No

Resident Name: _____

MG HSE WA

* Has the resident's physician recommended the use of this medical device? (Check Box)

- ☐ No
☐ Yes

* Please provide any additional comments _____

* Based on staff evaluation, is the resident safe to use the medical device? (Select one)

- ☐ Yes
☐ No

* The positive and negative aspects of medical device use have been discussed with the resident and/or family, and the resident and/or responsible parties are aware of the risks involved with the use of this medical device. (Select one)

- ☐ Yes
☐ No

Section: Transferring

* Transferring Level of Assistance (Select one)

☐ In House ☐ Outside Service ☐ Shared Responsibility

- ☐ Independent: Resident does not require assistance with transferring
- ☐ Minimal: Resident requires assistance with transfer and or positioning with verbal prompts/cues, no hands on assistance needed.
- ☐ Moderate: Resident requires occasional hands on assistance with transfers and or changes in position.
- ☐ Extensive: Resident requires frequent hands on assistance with transfers and or changes in position.
- ☐ Total: Resident requires routine hands on assistance with transfers and or changes in position.
- ☐ Not Applicable

* Transferring - Two Person Assist* (Select one)

☐ In House ☐ Outside Service ☐ Shared Responsibility

- ☐ Yes, Resident requires two person assist with transferring.
- ☐ No

* Transferring - Additional Services (Select one)

- ☐ Yes, Resident requires additional services with transferring.
- ☐ No

Resident Name: _____

MG HSE WA

- * **Transferring Enabling Devices and Methods - As a reminder please complete medical device section for residents requiring the use of devices below. (N/A in Alabama to slide board, transfer belt and trapeze)** (Select all that apply)

- ☐ Grab bar
- ☐ Slide board
- ☐ Transfer belt
- ☐ Trapeze
- ☐ Other - Resident has enabling device(s) used for transfers (see Service Plan)
- ☐ None Apply

Section: Pain and Discomfort Evaluation

Instructions: Complete upon initial evaluation and review during completion of re-evaluation/change of condition.

- * **Is the resident able to express pain?** (Select one)

- ☐ Yes
- ☐ No

- * **How does the resident express pain?** (Select all that apply)

- ☐ Verbal
- ☐ Facial grimacing
- ☐ Agitated facial expression
- ☐ Guarding
- ☐ Crying
- ☐ Moaning
- ☐ Physical aggression
- ☐ Change in appetite
- ☐ Other (provide details in Notes)
- ☐ None Apply

- * **Resident is currently experiencing pain or discomfort?** (Select one)

- ☐ Yes
- ☐ No

- * **Frequency** (Select one)

- ☐ Constant
- ☐ Daily
- ☐ Frequently
- ☐ Occasionally
- ☐ Rarely
- ☐ None Apply

Resident Name: _____

MG HSE WA

*** Type of Pain** (Select all that apply)

- ☐ Radiating
- ☐ Sharp
- ☐ Dull
- ☐ Shooting
- ☐ Aching
- ☐ Throbbing
- ☐ Stabbing
- ☐ Breakthrough
- ☐ Other (provide details in Notes)
- ☐ None Apply

*** Pain scale** (Select one)

- ☐ 0 - No pain
- ☐ 1 - 2 - Discomfort
- ☐ 3 - 4 - Mild Pain
- ☐ 5 - 6 - Moderate Pain
- ☐ 7 - 8 - Intense Pain
- ☐ 9 - 10 - Excruciating Pain

Location _____

Precipitating Factors _____

Relieving Factors _____

Section: Medication

*** Medication Level of Assistance** (Select one)

- ☐ Independent: Resident does not require assistance with medication administration
- ☐ Minimal: Resident requires minimal assistance (i.e. open containers or use of mediset) understands medication routine
- ☐ Moderate: Resident requires occasional assistance or cueing to follow medication routine or timely medication refill
- ☐ Extensive: Resident requires daily assistance to follow medication routine.
- ☐ Total: Resident is not able to take medication without assistance (not to exceed 30 days in ALA ALF).
- ☐ Not Applicable

*** Medication Pass Frequency** (Select one)

- ☐ 1-2 passes daily
- ☐ 3-4 passes daily
- ☐ 5+ passes daily
- ☐ Weekly pass
- ☐ None Apply

Resident Name: _____

MG HSE WA

*** Preferred Pharmacy Utilization** (Select all that apply)

- ☐ Resident uses community preferred pharmacy
- ☐ Resident uses approved non-preferred pharmacy (describe in Service Plan)
- ☐ Resident uses a non-preferred pharmacy
- ☐ Medications are not in the community preferred packaging (Not allowed in Alabama).
- ☐ Over-the-counter medications not provided by community preferred pharmacy
- ☐ None Apply

Section: Medication - Additional Information

*** Supervision** (Select one)

- ☐ Resident does not take medications.
- ☐ Resident can self-manage medications.
- ☐ Resident's family manages medications. (Not applicable in NV, ALA, or CA)
- ☐ Resident requires assistance/supervision for medications.

*** Counts** (Select one)

- ☐ No oral medications
- ☐ 1-5 oral medications
- ☐ 6-10 oral medications
- ☐ 11 or more oral medications

*** Crushed Order** (Select one)

- ☐ Yes, there is a crushed medication order (Not allowed in Alabama for ALF)
- ☐ No, there is not a crushed medication order

*** Special Prescribed Treatments** (Select one)

- ☐ No special prescribed treatments
- ☐ Resident receives assistance with special prescribed treatments 1 or 2 times/day
- ☐ Resident receives assistance with special prescribed treatments 3 or 4 times/day
- ☐ Resident receives assistance with special prescribed treatments 5 or more times/day

*** Medication Passes - Additional Services** (Select one)

- ☐ Yes, Resident requires additional services with medication passes.
- ☐ No

Section: Diabetes

*** Diabetes (select one)** (Select one)

- ☐ Resident does not have diabetes.
- ☐ Resident is insulin dependent diabetic.
- ☐ Resident is non-insulin dependent diabetic.

Resident Name: _____

MG HSE WA

* Diabetes - Level of Assistance (Select one)

☐ In House ☐ Outside Service ☐ Shared Responsibility

- ☐ Resident does not have diabetes.
- ☐ Resident is independent with diabetic management
- ☐ Minimum: Staff monitors and records blood sugar results/insulin injections. (Not applicable in NV)
- ☐ Moderate: Resident is able to complete blood sugar checks/insulin injections with staff setup. (Not applicable in NV)
- ☐ Total: Licensed staff to administer blood sugar checks/insulin injections. * (Outside provider required in NV)

* Does the resident have diabetes which is managed by outside service (only applicable in Nevada)? (Select one)

- ☐ Yes
- ☐ No or Not Applicable

Section: Self-Administration of Medication Evaluation

Instructions: Complete upon initial evaluation and review during completion of re-evaluation/change of condition. Otherwise, select not applicable. N/A for all Garden House residents.

* Does the resident currently self-administer his or her medication? (Select one)

- ☐ Yes
- ☐ No
- ☐ Sometimes
- ☐ N/A, resident does not currently take medications.

* Can the resident state what medication they take (i.e., prescribed, over-the-counter, supplements, etc.)? The resident may use a list or read off of packaging when responding to this question. (Select one)

- ☐ Yes
- ☐ No
- ☐ Sometimes

* Can the resident state what the medication is used for? (Select one)

- ☐ Yes
- ☐ No
- ☐ Sometimes

* Can the resident state when they take each medication? (Select one)

- ☐ Yes
- ☐ No
- ☐ Sometimes

* Can the resident open a container? (Select one)

- ☐ Yes
- ☐ No

* Can the resident read the medication label? (Select one)

- ☐ Yes
- ☐ No

Resident Name: _____

MG HSE WA

* Can the resident identify the proper medication dosage? (Select one)

- ☐ Yes
☐ No
☐ Sometimes

* Is there a reported history of non-compliance with a medication regimen? (Select one)

- ☐ Yes
☐ No

* Can the resident report adverse affects of medication? (Select one)

- ☐ Yes
☐ No
☐ Sometimes

* Physician's order obtained for self-administration of medication? Note that this is not required in the state of Washington. (Select one)

- ☐ Yes
☐ No
☐ N/A

* If family is assisting, has the Family Assistance form been completed? This form is only required in WA state. (Select one)

- ☐ Yes
☐ No
☐ N/A

Section: Self-Administration of Medication Evaluation Score

* Total Score: Self-Administration of Medication Evaluation (Select one)

- ☐ 0 - 4 Resident demonstrates ability to self-administer medications.
☐ 5 - 15 Resident does not demonstrate the ability to self-administer medications and requires assistance.

Section: Skin Evaluation

* Overall skin condition (Select one)

- ☐ Normal
☐ Abnormal
☐ Other (describe other in Service Plan)

* Lotion to Extremities (Select one)

- ☐ Not Applicable
☐ Resident requires assistance with lotion to extremities

☐ In House ☐ Outside Service ☐ Shared Responsibility

* Describe location and type of skin marking(s) (i.e., scars, birthmarks, healed surgery sites, tattoos, etc.) _____

Resident Name: _____

MG HSE WA

- * Describe location and type of active skin issue(s) (i.e., open areas, broken skin, rashes, bruising, etc.) _____

- * Resident has an active wound (Select one)

- ☐ Yes (describe below)
☐ No

Describe location, type, and date of onset of active wound(s) _____

Section: Bathing

- * Bathing Level of Assistance (Select one)

- ☐ Independent: Resident does not require assistance with bathing
☐ Minimal: Resident can bathe without physical assistance but may require reminding or standby assistance
☐ Moderate: Resident requires assistance with bathing, requires assistance or cueing with parts of bathing including assistance getting in/out of tub/shower
☐ Extensive: Resident requires hands on assistance with participation by the resident to complete task
☐ Total: Resident is dependent on others to provide complete bath, including shampoo

- * Bathing Frequency (Select one)

- ☐ 1-2 times weekly
☐ 3-4 times weekly
☐ 5-6 times weekly
☐ Daily
☐ None Apply

- * Bathing Preferences (Select all that apply)

- ☐ Prefers baths
☐ Prefers showers
☐ Other (see Service Plan)
☐ None Apply

- * Bathing Enabling Devices and Methods (Select all that apply)

- ☐ Shower chair
☐ Bench
☐ Grab bar
☐ Hand held shower
☐ Bath lift
☐ Other (see Service Plan)
☐ None Apply

Resident Name: _____

MG HSE WA

Section: Bathing - Additional Information

* **Bathing - Supervision** (Select one)

- ☐ Resident is Independent with bathing.
- ☐ Resident requires assistance/supervision from Outside Service or Family for bathing.
- ☐ Resident requires assistance/supervision from staff for bathing.

* **Number of Spa Baths per Week** (Select one)

- ☐ 0 or Not Applicable
- ☐ 1
- ☐ 2
- ☐ 3
- ☐ 4
- ☐ 5
- ☐ 6
- ☐ 7

* **Bathing - Two Person Assist*** (Select one)

- ☐ Yes
- ☐ No

☐ In House ☐ Outside Service ☐ Shared Responsibility

* **Bathing - Additional Services** (Select one)

- ☐ Yes, Resident requires additional services with bathing.
- ☐ No

Section: Grooming/Personal Hygiene

* **Grooming/Personal Hygiene Level of Assistance** (Select one)

- ☐ Independent: Resident does not require assistance with grooming/personal hygiene
- ☐ Minimal: Resident may manage grooming/personal hygiene but requires verbal reminders/prompts/cues
- ☐ Moderate: Resident performs grooming/personal hygiene but requires physical assistance to complete task
- ☐ Extensive: Caregiver performs most grooming/personal hygiene but resident is able to assist
- ☐ Total: Resident is dependent on others to provide all grooming/personal hygiene needs

☐ In House ☐ Outside Service ☐ Shared Responsibility

* **Does resident have their own teeth?** (Select one)

- ☐ Yes
- ☐ No

Resident Name: _____

MG HSE WA

*** Grooming/Personal Hygiene Enabling Devices and Methods** (Select all that apply)

- ☐ Glasses
- ☐ Contact lenses
- ☐ Artificial eye
- ☐ Hearing aid(s)
- ☐ Full dentures
- ☐ Partial dentures
- ☐ Upper dentures
- ☐ Lower dentures
- ☐ Bridge
- ☐ Other - Resident has enabling device(s) used for grooming/personal hygiene (see Service Plan)
- ☐ None Apply

*** Grooming - Additional Services** (Select one)

- ☐ Yes, Resident requires additional services with grooming.
- ☐ No

Section: Dressing

*** Dressing Level of Assistance** (Select one)

☐ In House ☐ Outside Service ☐ Shared Responsibility

- ☐ Independent: Resident does not require assistance with dressing
- ☐ Minimal: Resident can dress/undress and select clothing but may need to be reminded/supervised
- ☐ Moderate: Resident can dress/undress and select clothing with physical assistance
- ☐ Extensive: Resident requires assistance with dressing, caregiver dresses/undresses and selects clothing but resident is able to assist in task
- ☐ Total: Resident is dependent upon others to do all dressing/undressing

*** Dressing Enabling Devices and Methods** (Select all that apply)

- ☐ Button hook
- ☐ Dressing stick
- ☐ Shoe horn
- ☐ Quick zips
- ☐ Other - Resident has enabling device(s) used for dressing (see Service Plan)
- ☐ None Apply

*** Dressing - Additional Services** (Select one)

- ☐ Yes, Resident requires additional services with grooming.
- ☐ No

*** Dressing - Two Person Assist*** (Select one)

- ☐ Yes, Resident requires two person assistance with dressing.
- ☐ No

*** Wake Up Call** (Select one)

☐ In House ☐ Outside Service ☐ Shared Responsibility

- ☐ Not Applicable
- ☐ Wake up call
- ☐ In room wake up call

Resident Name: _____

MG HSE WA

* Does resident require assistance with TED Hose? (Select one)

☐ In House ☐ Outside Service ☐ Shared Responsibility

- ☐ No, resident does not wear Ted Hose
☐ Yes, resident requires assistance with TED Hose
☐ Resident wears Ted Hose Independently

Section: Toileting

* Toileting Level of Assistance (Select one)

☐ In House ☐ Outside Service ☐ Shared Responsibility

- ☐ Independent: Resident does not require assistance with toileting
☐ Minimal: Resident requires verbal prompts/cues for toileting tasks.
☐ Moderate: Resident requires stand by assistance for toileting tasks.
☐ Extensive: Resident requires physical assistance with parts of toileting tasks.
☐ Total: Resident requires physical assistance with all tasks related to toileting. May require assistance with closed drainage system/catheters/ostomy.

* Resident is continent of bladder? (Select one)

- ☐ Yes
☐ Usually
☐ No

* Resident is continent of bowel? (Select one)

- ☐ Yes
☐ Usually
☐ No

* Toileting Enabling Devices and Methods (N/A in Alabama for Slide Board and Transfer Belts) (Select all that apply)

- ☐ Adult Briefs
☐ Adult Pull-up/protective underwear
☐ Bedpan
☐ Bedside commode
☐ Catheter-external/condom
☐ Catheter-indwelling
☐ Colostomy
☐ Grab bar
☐ Ileostomy
☐ Mattress protector
☐ Raised seat
☐ Slide board
☐ Transfer belt
☐ Ureterostomy
☐ Urinal
☐ Urinary pad
☐ Other - Resident has enabling device(s) used for toileting (see Service Plan)
☐ None Apply

Resident Name: _____

MG HSE WA

- * **Toileting - Two Person Assist*** (Select one) ☐ In House ☐ Outside Service ☐ Shared Responsibility
- ☐ Yes, Resident requires two person assistance with toileting.
- ☐ No

- * **Toileting - Additional Services** (Select one) ☐ In House ☐ Outside Service ☐ Shared Responsibility
- ☐ Yes, Resident requires additional services with toileting.
- ☐ No

Section: Incontinence

- * **Incontinence Level of Assistance** (Select one) ☐ In House ☐ Outside Service ☐ Shared Responsibility
- ☐ Not Applicable: Resident has no reported incontinence
- ☐ Independent: Resident self manages incontinence
- ☐ Minimal: Resident requires minimal assistance with incontinence (personal hygiene, set-up, cueing, ordering of incontinence supplies)
- ☐ Moderate: Resident requires moderate assistance with incontinence (personal hygiene, cueing, clothing assistance, assist with ordering of incontinence supplies)
- ☐ Extensive: Resident requires extensive assistance with incontinence (personal hygiene, pericare, cueing, clothing assistance, assist with ordering of incontinence supplies)

- * **Incontinence - Two Person Assist*** (Select one)
- ☐ Yes
- ☐ No

- * **Incontinence - Additional Services** (Select one)
- ☐ Yes, Resident requires additional services with incontinence needs.
- ☐ No

Section: Dining Services

- * **Diet** (Select all that apply) ☐ In House ☐ Outside Service ☐ Shared Responsibility
- ☐ Regular
- ☐ CCHO - Liberal Diabetic
- ☐ Heart Healthy
- ☐ Consistency - Mechanical Soft
- ☐ Consistency - Pureed
- ☐ Thickened Liquids - Nector
- ☐ Thickened Liquids - Honey
- ☐ Finger Foods

- * **Resident Preferences** (Select one)
- ☐ Resident will determine his/her own food preferences. (See Service Plan)
- ☐ Resident requires assistance with determining his/her food preferences. (See Service Plan)

Resident Name: _____

MG HSE WA

* **Additional Meal Considerations** (Select all that apply)

- ☐ Problems with chewing
- ☐ Problems with swallowing
- ☐ Loose teeth
- ☐ Broken teeth
- ☐ Decaying teeth
- ☐ Recent unintended weight loss
- ☐ Recent unintended weight gain
- ☐ None Apply

* **Meal Consumption Level of Assistance** (Select one) ☐ In House ☐ Outside Service ☐ Shared Responsibility

- ☐ Independent: Resident does not require assistance with dining services.
- ☐ Minimal: Resident can feed self, chew and swallow foods, however needs reminding/cueing to maintain adequate intake
- ☐ Moderate: Resident requires cutting up of food, opening cartons/packages; may need encouragement to select menu items
- ☐ Extensive: Resident requires regular encouragement to select menu items compliant with ordered diet; may require assistance with feeding appliances.
- ☐ Total: Resident must be fed by mouth by another person or gastronomy (Not applicable in NV)

* **Meal Consumption Enabling Devices and Methods** (Select all that apply)

- ☐ Plate guard
- ☐ Adaptive utensil (spoon and/or fork)
- ☐ Raised lip plate
- ☐ Adaptive cup
- ☐ Tube feeding (Not applicable in NV)
- ☐ Other - Resident has enabling device(s) used for meal consumption (see Service Plan)
- ☐ None Apply

* **Meal Consumption - Additional Services** (Select one)

- ☐ Yes, Resident requires additional services with meal consumption assistance.
- ☐ No

* **Meal Preparation** (Select one) ☐ In House ☐ Outside Service ☐ Shared Responsibility

- ☐ Not applicable: Resident does not require meal preparation assistance in room.
- ☐ Resident requests meal preparation assistance in room (1-2x weekly).
- ☐ Resident requests meal preparation assistance in room (1x daily).

* **Meal Delivery** (Select one) ☐ In House ☐ Outside Service ☐ Shared Responsibility

- ☐ Not Applicable - Resident does not receive meal delivery OR receives meals through Hospice.
- ☐ Resident requests meal delivery (1-2x weekly)
- ☐ Resident requests meal delivery (1x daily)
- ☐ Resident requests meal delivery (2x daily)
- ☐ Resident requests meal delivery (3x daily)

Resident Name: _____

MG HSE WA

Section: Cooking Evaluation

Instructions: AL and IL only. Complete upon initial evaluation and review during completion of re-evaluation/change of condition. Otherwise, select not applicable.

* Can resident turn stove burners on and off? (Select one)

- ☐ Yes
☐ No
☐ N/A

* Does resident check oven for contents prior to turning on? (Select one)

- ☐ Yes
☐ No
☐ N/A

* Can resident turn oven on and off? (Select one)

- ☐ Yes
☐ No
☐ N/A

* Can resident demonstrate use of microwave by heating a cup of water? (Select one)

- ☐ Yes
☐ No
☐ N/A

* Does resident use appliances appropriately? (Select one)

- ☐ Yes
☐ No
☐ N/A

* Does resident have a diagnosis of Cognitive Impairment? (Select one)

- ☐ Yes
☐ No
☐ N/A

* Based upon staff evaluation, is the resident safe to cook unattended? (Select one)

- ☐ Yes
☐ No
☐ N/A

* If no, has the stove/oven breaker been turned off? (Select one)

- ☐ Yes
☐ No
☐ N/A

Notes (Select one)

- ☐ Select to add notes

Notes: _____

Resident Name: _____

MG HSE WA

Section: Activities

* **Activities Level of Assistance** (Select one) ☐ In House ☐ Outside Service ☐ Shared Responsibility

- ☐ None: Resident does not require assistance with activities.
- ☐ Minimal: Resident may require occasional verbal reminders for scheduled activities.
- ☐ Moderate: Resident requires frequent verbal and may require physical assistance with activities.
- ☐ Extensive: Resident requires daily reminders and physical assistance for activities.
- ☐ Total: Resident requires physical or cognitive assistance with any task.

* **Preferred Activity Settings** (Select all that apply)

- ☐ 1 on 1
- ☐ Alone
- ☐ Large group
- ☐ Small groups
- ☐ Activity room or dining room
- ☐ Own room
- ☐ Offsite
- ☐ Outside at community
- ☐ Other (see Service Plan)
- ☐ None Apply

* **General Activity Preferences** (Select all that apply)

- ☐ Cards/other games
- ☐ Crafts/arts
- ☐ Exercise/sports
- ☐ Dancing
- ☐ Music
- ☐ Reading/writing
- ☐ Spiritual/religious
- ☐ Trips/shopping
- ☐ Walking/wheeling outdoors
- ☐ Watching TV
- ☐ Gardening or plants
- ☐ Talking or conversing
- ☐ Helping others
- ☐ Doing chores around the house/community
- ☐ Other (see Service Plan)
- ☐ None Apply

Section: Routines and Preferences

Resident Preferences (Select one)

- ☐ Resident has other preferences or routines (See Service Plan)
- ☐ Resident does not have other preferences or routines

Section: Emergency and Evacuation

*** Emergency Response Level of Assistance** (Select one)

- ☐ Independent: Resident does not require assistance with using the community response system
- ☐ Minimal: Resident requires occasional reminding with how to use the emergency response system
- ☐ Moderate: Resident requires frequent reminding with how to use the emergency response system
- ☐ Extensive: Resident requires repeated reminding with how to use the emergency response system, ongoing training, and may need physical assistance
- ☐ Total: Resident is unable to utilize the emergency response system (see Service Plan)

*** Resident trained on and demonstrates how to activate emergency call system** (Select one)

- ☐ Yes
- ☐ No/Unable

*** Emergency Response Enabling Devices and/or Methods** (Select all that apply)

- ☐ Emergency Pull Cord
- ☐ Pendant Call System
- ☐ Other - Resident has enabling device(s) used for emergency response (see Service Plan)
- ☐ None Apply

*** Evacuation Level of Assistance** (Select one)

- ☐ Independent: Resident does not require assistance with evacuation or to request emergency assistance
- ☐ Minimal: Resident requires supervision and/or verbal cueing to evacuate residence or to request emergency assistance
- ☐ Moderate: Resident requires some physical assistance to evacuate residence or to request emergency assistance
- ☐ Extensive: Staff must assist resident to evacuate residence or to request emergency assistance
- ☐ Total: Resident requires physical assistance to evacuate residence or to request emergency assistance

*** Evacuation Enabling Devices and Methods (N/A in Alabama to slide board, transfer belt and trapeze)** (Select all that apply)

- ☐ Pet evacuation required
- ☐ Brace/orthotic appliance
- ☐ Cane (regular, quad)
- ☐ Crutch
- ☐ Gait belt
- ☐ Grab bar
- ☐ Prosthesis
- ☐ Slide board
- ☐ Transfer belt
- ☐ Trapeze
- ☐ Walker (no wheels, wheeled, seat attached)
- ☐ Wheelchair
- ☐ Scooter/Electric Wheelchair/Motorized Device
- ☐ Other - Resident has enabling device(s) used for Evacuation (see Service Plan)
- ☐ None Apply

Resident Name: _____

MG HSE WA

- * Can resident leave the community unsupervised? If the answer is no, please provide the plan in the service plan. (Check Box)

☐ No
☐ Yes

- * Is the resident able to exit the building without a verbal cue? If not, please document the plan in the service plan. (Check Box)

☐ No
☐ Yes

Section: Nursing Oversight

- * Nursing Oversight Level of Assistance (Select one)

- ☐ Independent: Resident does not require any additional nursing oversight or can manage services on own
- ☐ Minimal: Resident requires occasional/monthly nursing oversight such as ordering of supplies, scheduling of appointments, monthly pacer checks and other services including communication with multiple health care providers.
- ☐ Moderate: Resident requires bimonthly nursing oversight such as ordering of supplies, scheduling of appointments, coordination with physician including communication with multiple health care providers
- ☐ Extensive: Resident requires weekly nursing oversight such as scheduling of appointments, regular coordination with physician including regular communication with multiple health care providers.
- ☐ Total: Resident requires frequent nursing oversight.

- * Nursing Oversight - Additional Services (Select one)

- ☐ Yes, resident requires additional services with nursing oversight.
- ☐ No

- * Nursing Services Outside Services (Select all that apply)

- ☐ Physical Therapy
- ☐ Occupational Therapy
- ☐ Speech/Swallow Therapy
- ☐ Social Worker/Case Manager
- ☐ Hospice/Palliative Care
- ☐ Registered Nurse (RN) or Licensed Practical/Vocational Nurse (LP/VN)
- ☐ Home Health Aide
- ☐ Private Duty Caregiver (PDA)
- ☐ Other (specify in Service Plan)
- ☐ None Apply

Notes: _____

Resident Name: _____

MG HSE WA

*** Long Term Care Insurance** (Select one)

- ☐ Not Applicable
- ☐ Minimal time needed for completion of Long Term Care Insurance
- ☐ Moderate time needed for completion of Long Term Care Insurance
- ☐ Extensive time needed for completion of Long Term Care Insurance

Section: Other Additional Services

*** Additional Services (other than outside care). Additional services are being provided and are not captured in the evaluation above. Please provide detail in service plan.** (Select all that apply)

- ☐ No Additional Services
- ☐ 25 - Minimal Level of Assistance
- ☐ 75 - Moderate Level of Assistance
- ☐ 125 - Extensive Level of Assistance
- ☐ 175 - Maximum Level of Assistance

*** Outside Services. The Resident has hired an outside service provider to complete tasks as listed in this evaluation. The points below account for Merrill Gardens' coordination and oversight of these outside services being provided in the community. Select the level of care points and provide detail within the service plan.** (Select all that apply)

- ☐ No Additional Services
- ☐ 25 - Minimal Level of Assistance
- ☐ 75 - Moderate Level of Assistance
- ☐ 125 - Extensive Level of Assistance
- ☐ 175 - Maximum Level of Assistance

Section: Housekeeping and Laundry

*** Additional Housekeeping and/or Laundry Services over and above what is included in Rent required?** (Select one)

- ☐ Independent (AL and GH): resident does not receive additional housekeeping or laundry services outside of what is included in rent.
- ☐ Additional Services (AL and GH): resident receives additional housekeeping or laundry services outside of what is included in rent.

Resident Name: _____

MG HSE WA

*** Additional Housekeeping and Laundry Services Required:** (Select all that apply)

- ☐ Personal Laundry (AL and GH) 1-2x per week
- ☐ Personal Laundry (AL and GH) 3-4x or more times per week
- ☐ Personal Laundry (AL and GH) 5-7x times per week
- ☐ Daily Housekeeping (AL only): trash
- ☐ Daily Housekeeping: bathroom
- ☐ Daily Housekeeping: bed making
- ☐ Daily Housekeeping: dust
- ☐ Daily Housekeeping: vacuum
- ☐ No Additional Housekeeping or Laundry Services

Section: Pet Care

*** Pet Care Level of Assistance** (Select one)

- ☐ Resident does not have a pet
- ☐ Resident does have a pet but can independently manage pet needs
- ☐ Resident requires occasional reminders to care for pet
- ☐ Resident requires staff to assist in providing pet care (1-2x weekly)
- ☐ Resident requires staff to assist in providing pet care (1x daily)
- ☐ Resident requires staff to assist in providing pet care (2x daily)
- ☐ Resident requires staff to assist in providing pet care (3x daily)

Section: Negotiated Service Agreements

* Negotiated Service Agreements _____
(indicate if none apply)

Section: HABITS - Washington

* Describe resident's current alcohol use and/or history (if applicable) _____

Describe resident's current tobacco use and/or history (if applicable) _____

Describe resident's current recreational drug use and/or history (if applicable) _____

Smoking Safety Provisions (if applicable) _____

Section: HEALTH - Washington

Resident Name: _____

MG HSE WA

* Describe respiratory condition (including airway, breathing, right and left sounds) _____

* Describe cardiovascular health (include any dizziness, shortness of breath, edema, cyanosis, etc.) _____

* Describe skin health (including temperature, turgor, etc.) _____

* **Skin Condition(s)** (Select all that apply)

- ☐ Dry skin
- ☐ Flaky skin
- ☐ Rash
- ☐ Lesion
- ☐ Stasis ulcer
- ☐ Pressure ulcer
- ☐ Scars
- ☐ Burns
- ☐ Ecchymosis
- ☐ Abrasion
- ☐ Wound
- ☐ Decubitus wound
- ☐ Dressing(s)
- ☐ Stoma
- ☐ Colostomy bag
- ☐ None Apply

Section: RESIDENT CHARACTERISTIC ROSTER - Washington

* **Nursing Services (services only a licensed nurse can provide)** (Select all that apply)

- ☐ O - resident receiving Ostomy care
- ☐ T - resident receiving Tube feeding
- ☐ I - resident receiving Injections
- ☐ ND - resident receiving Nurse Delegation
- ☐ None Apply

* **Medication** (Select all that apply)

- ☐ I - resident assessed as Independent with their medication
- ☐ A - resident assessed as needing medication assistance
- ☐ AD - resident assessed as needing medication administration
- ☐ F - resident receiving Family assistance with medications
- ☐ None Apply

Resident Name: _____

MG HSE WA

* **Mobility / Falls / Ambulation Devices** (Select all that apply)

- ☐ A - resident requires Assistance with transfers or cannot ambulate independently without assistance from staff or assistive devices
- ☐ F - resident experienced a Fall within the last 30 days
- ☐ D - resident uses a Device to assist with ambulation
- ☐ None Apply

* **Behavior / Psychosocial Issues** (Select all that apply)

- ☐ X - resident shows or has behaviors such as those requiring special training or assistance increasing the amount of time staff needs to assist resident
- ☐ None Apply

* **Dementia / Alzheimer's / Cognitive Impairment** (Select all that apply)

- ☐ X - resident shows or has behaviors requiring special training or assistance increasing the amount of time staff needs to assist resident
- ☐ None Apply

* **Exit Seeking / Wandering** (Select all that apply)

- ☐ ES - resident has shown Exit Seeking behaviors
- ☐ W - resident has shown Wandering behaviors
- ☐ None Apply

* **DD / Mental Health** (Select all that apply)

- ☐ DD - resident has a Developmental Disability case manager
- ☐ MH - resident receives Mental Health services and/or has a mental health case manager
- ☐ None Apply

* **Language / Communication Issues / Deafness / Hearing Issues** (Select all that apply)

- ☐ X - resident has a language or communication issue which requires additional staff support
- ☐ HI - resident is Hearing Impaired
- ☐ D - resident is Deaf
- ☐ None Apply

* **Vision Deficit / Blindness** (Select all that apply)

- ☐ X - resident is blind or has severe vision deficit which requires additional staff support
- ☐ None Apply

* **Assist with ADLs** (Select one)

- ☐ I - resident assessed as Independent
- ☐ MIN - resident assessed as needing MINimal assistance with ADLs such as cueing, reminders, supervision, and/or encouragement
- ☐ MOD - resident assessed as needing MODerate assistance with ADLs such as guiding, standby assistance for transfers, or ambulation, bathing, and toileting
- ☐ MAX - resident assessed as needing MAXimum assistance with ADLs such as needing a one person or two person transfer, resident was incontinent of bowel or bladder and required staff to assist with care; resident needed assistance with turning, sitting up or laying down, staff must physically turn the resident every two hours

Resident Name: _____

MG HSE WA

* **Wounds / Skin Issue** (Select all that apply)

- ☐ P - resident has a Pressure ulcer
- ☐ S - resident has a Stasis wound
- ☐ W - resident has a Wound or skin issue other than pressure or stasis ulcer
- ☐ None Apply

* **Incontinent / Appliance (catheter) / Dialysis** (Select all that apply)

- ☐ UI - resident Incontinent of bladder and/or bowel
- ☐ C - resident has Catheter
- ☐ D - resident requires Dialysis
- ☐ None Apply

* **Special Dietary Needs / Scheduled Snacks** (Select all that apply)

- ☐ X - resident requires a special prescribed diet
- ☐ None Apply

* **Weight Loss / Weight Gain** (Select all that apply)

- ☐ WL - resident has had more than a 3 - 5 pound Weight Loss within last 60 days
- ☐ WG - resident has had more than a 3 - 5 pound Weight Gain within the last 60 days
- ☐ None Apply

* **Medical Devices** (Select all that apply)

- ☐ X - resident receives dialysis treatments
- ☐ M - if part of resident's care is the use of side rails, transfer poles, chair, bed alarms, belt restraints
- ☐ None Apply

* **Pay Status** (Select all that apply)

- ☐ P - all or part of resident's care is paid by the resident or their family
- ☐ S - all or part of resident's care is paid for by the state
- ☐ None Apply

* **Recent Hospitalization** (Select all that apply)

- ☐ X - resident has been hospitalized within the last 60 days
- ☐ None Apply

* **Oxygen / Respiratory Therapy** (Select all that apply)

- ☐ X - resident receives oxygen and/or respiratory therapy or treatments
- ☐ None Apply

* **Home Health / Hospice / Private Caregiver** (Select all that apply)

- ☐ HH - resident receives Home Health services
- ☐ HOS - resident receives Hospice services
- ☐ P - resident receives care from Private caregiver
- ☐ None Apply

Resident Name: _____

MG HSE WA

Section: RN NURSE DELEGATION - Washington

* **Resident requires Nurse Delegation:** (Select one)
(if yes, make sure to select appropriate answer under applicable sections throughout evaluation).

- ☐ Yes (Resident will need to be charged through an ancillary fee)
☐ No

* **Current Nurse Delegations services:** (Select all that apply)

- ☐ Administration of Oral Medications
☐ Administration of Oral Medications - PRN
☐ Administration of Rectal Suppository
☐ Management of Oxygen Administration
☐ Administration of Oral Inhalation Therapy
☐ Administration of Ear Drops
☐ Administration of Eye Drops
☐ Administration of Eye Ointment
☐ Administration of Nose Spray
☐ Management of Indwelling Catheter
☐ Administration of Vaginal Cream or Suppository
☐ N/A no Nurse Delegation at the time of evaluation

Notes: _____

Signatures:

Resident **Date:**

Community Representative **Date:**

Responsible Party **Date:**

Community Representative **Date:**



Mini-Mental State Examination (MMSE)

Baseline / 5 Year follow-up (please circle as appropriate)

	Score
1. Orientation Say - Can you tell me the date? Ask specifically for any items omitted (day, date, month, season, year). Allow flexibility when the season changes. Score one point for each correct answer. Score 0-5 Say - Can you tell me the name of this place? What town/city, county, country are we in? What floor of the building are we on? (check meaning of first and ground floor). Score one point per correct answer. Score 0-5	
2. Registration Say - I would like to test your memory. I want you to remember three things - apple, table, penny (say items clearly and slowly allowing one second between each item). Say - Can you repeat them? First repetition determines score, one point for each exactly correct answer. Score 0-3	
3. Attention and Calculation Say - Start with 100 and keep taking 7 away until I tell you to stop. (Continue to 5 subtractions). Score a point when patient successfully subtracts seven even if previous number was wrong. If patient cannot or will not perform the task, test reverse spelling. Say - I would like you to spell 'WORLD' backwards. Score the number of letters in the correct order (D=1, L=1, R=1, O=1, W=1). Score 0-5	
4. Recall Say - Can you tell me the three things that I asked you to remember? (apple, table and penny). Allow ten seconds for reply. Give one point for each exactly correct answer. Score 0-3	
5. Naming Accurate naming is required; descriptions of function or approximate answers are unacceptable. Show the patient a wristwatch and ask - What is this? Score one point for either watch, wristwatch or time-piece. Score 0-1 Show the patient a pencil and ask - What is this? Score one point for pencil only. If approximate answer is given say - Can you think of another word for this? Score 0-1	
6. Repetition Say - Listen carefully and repeat what I say, 'No ifs ands or buts'. Read phrase slowly and clearly enunciating all the S's. Score one point for correct phrase. Score 0-1	
7. Three stage command Say - Take this piece of paper in your right hand, fold it in half using both hands and put it on the floor. Hand A4 piece of paper to patient's mid-line, allow 30 seconds, score one point for each correct stage completed in the correct order. Do not coach or repeat instructions. Score 0-3	
8. Reading Comprehension Show the patient the statement "Close your eyes", (written overleaf). Say - Read this and do what it says. Repeat instructions if necessary. Score one point if patient closes eyes. Score 0-1	
9. Writing Give the patient a pen and the reverse side of this sheet. Say - Write a sentence on this piece of paper, (anything will do as long as it makes sense). If the patient does not appear to understand, repeat instructions. Score one point if there is a subject and a verb, correct spelling, grammar and punctuation are not necessary. Allow 30 seconds to complete task. Score 0-1	
10. Praxis Show the intersecting pentagons overleaf. Say-Copy this shape. Score one point if there are five sides and five angles on each pentagon, and the overlap forms a diamond. Ignore tremor and rotation. Allow up to one minute and patient may be allowed multiple attempts. Score 0-1	
TOTAL SCORE	
CONSCIOUSNESS Estimate by marking on the line patient's conscious level on a continuum from fully alert on the left to coma on the right. This does not contribute to the total score. Alert Drowsy Stupor Coma	
Name of person administering MMSE _____ Date _____	

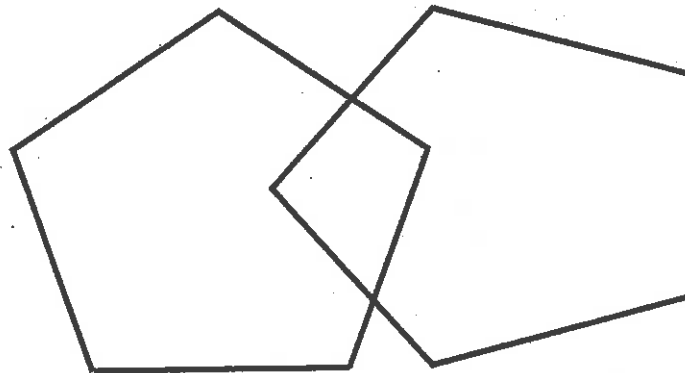
Please complete the following
PD MED Trial Number

Patient Initials:
Date of Birth: / /
Date Completed: / /

Trial office use only

Date Sent: / /
Date Received: / /
Date Entered: / /

Close your eyes



MMSE - Instructions For Administration

Make the patient comfortable, establish rapport, and praise success.

Ask questions in the order listed and score immediately.

Total possible score is 30.

ORIENTATION

- (1) Ask for today's date; then ask for specific parts omitted; e.g., "Can you also tell me what season it is?"
- (2) Ask where patient lives (street and county) and where patient is now (city, state, present location).

REGISTRATION

Ask patient if you may test his/her memory. Then say the name of 3 objects (apple, penny, table) clearly and slowly, about one second for each. After you have said all 3, ask him/her to repeat them. This first repetition determines the score (0-3), but keep saying them until he/she can repeat all 3, up to 5 trials. Otherwise, recall cannot be meaningfully tested.

ATTENTION AND CALCULATION

- (1) Ask patient to begin with 100 and subtract 7, and keep subtracting 7 from each answer until you say stop (after 5 subtractions). Score total number of correct answers (e.g., 93, 80, 73, 66, 59 = "4" correct).
- (2) Ask the patient to spell "WORLD"; if correct, ask patient to spell it backwards. Score the number of letters in correct order (e.g., DLROW = 5; DLORW = 3)
- (3) Score whichever (1) or (2) is the highest.

RECALL

Ask patient if he/she can recall the 3 words you previously asked him/her to remember. Score 0-3.

LANGUAGE

Naming: Show patient a wristwatch and ask what it is; repeat for a pencil or pen. Score 0-2

Repetition: Ask patient to repeat: "No ifs ands or buts". Allow only one trial. Score 0-1

3-Stage Command: Ask patient to do exactly as you say: "Take this paper in your right hand, fold it in two, and drop it on the floor." Score 0-3; i.e., one point for each part correctly executed.

Reading: Ask patients to read the sentence "CLOSE YOUR EYES" and do what it says. Score 1 point if he/she actually closes his/her eyes.

Writing: Ask patient to write a sentence for you. It must contain a subject and verb and be sensible. Correct grammar and punctuation are not necessary.

Copying: Ask patient to copy the figure of intersecting pentagons exactly as it is. All 10 angles must be present and 2 must intersect to form a 4-sided figure to score 1 point. Tremor and rotation are ignored.

"Simple" Clock-Drawing Test

Directions: Ask patient to draw face of a clock and put the numbers in the correct position. Then ask patient to draw in the hands at a particular time (e.g., "twenty minutes after eight"). Scoring is 0-4 as indicated below. Any cut off score is subjective and arbitrary. It is unlikely that a cognitively impaired person will produce a perfect clock. A low score indicates need for further evaluation.

Draws closed circle:	Score 1 point
Includes all 12 correct numbers:	Score 1 point
Places numbers in correct positions:	Score 1 point
Places hands in correct positions:	Score 1 point

Physical Self-Maintenance Scale (PSMS)

Activities of Daily Living

Patient's Name _____

Date _____

Rated by _____

Numbers one through five in each category represent worsening states of function. Choose the number that best describes the patient's functional status. Scores in all six categories should then be totaled. The higher the final score, the greater the degree of impairment.

		Score
A. Toileting	1. Cares for self at toilet completely, no incontinence. 2. Needs to be reminded or needs help in cleaning self, or has rare (weekly at most) accidents. 3. Soiling or wetting while asleep more than once a week. 4. Soiling or wetting while awake more than once a week. 5. No control of bowels or bladder.	_____
B. Feeding	1. Eats without assistance. 2. Eats with minor assistance at mealtimes and/or with special preparation of food, or help in cleaning up after meals. 3. Feeds self with moderate assistance and is untidy. 4. Requires extensive assistance for all meals. 5. Does not feed self at all and resists efforts of others to feed him/her.	_____
C. Dressing	1. Dresses, undresses, and selects clothes from own wardrobe. 2. Dresses and undresses self with minor assistance. 3. Needs moderate assistance in dressing or selection of clothes. 4. Needs major assistance in dressing, but cooperates with efforts of others to help. 5. Completely unable to dress self and resists efforts of others to help.	_____
D. Grooming (neatness, hair, nails, hands, face, clothing)	1. Always neatly dressed, well groomed, without assistance. 2. Grooms self adequately with occasional minor assistance, eg, shaving. 3. Needs moderate and regular assistance or supervision in grooming. 4. Needs total grooming care, but can remain well groomed after help from others. 5. Actively negates all efforts of others to maintain grooming.	_____
E. Physical Ambulation	1. Goes about grounds or city. 2. Ambulates within residence or about one block distance. 3. Ambulates with assistance of (check one) a () another person b () railing c () cane d () walker e () wheelchair—gets in and out without help f () wheelchair—needs help in getting in, out. 4. Sits unsupported in chair or wheelchair, but cannot propel self without help. 5. Bedridden more than half the time.	_____
F. Bathing	1. Bathes self (tub, shower, sponge bath) without help. 2. Bathes self with help in getting in and out of tub. 3. Washes face and hands only, but cannot bathe rest of body. 4. Does not wash self but is cooperative with those who bathe him/her. 5. Does not try to wash self, and resists efforts to keep him/her clean.	_____

To track patient's cognitive and functional status, record the baseline and follow-up scores of the MMSE and PSMS in the chart provided:

	MMSE	PSMS	DATE
Baseline			
Follow-up			
Follow-up			
Follow-up			
Follow-up			
Follow-up			

Total Score _____



Change in Services

Complete only the sections that are to be changed

Resident _____ Apartment _____

Apartment Transfer

Current Apartment _____ Rent \$ _____ New Apartment _____ Rent \$ _____ Effective Date _____

<u>Common Items or Services</u>	<u>Date</u>	<u>Monthly Fee</u>	<u>Date</u>
Parking # _____ Add Service _____	_____	\$ _____	Discontinue Effective _____
Storage # _____ Add Service _____	_____	\$ _____	Discontinue Effective _____
Additional Laundry Add Service _____	_____	\$ _____	Discontinue Effective _____
Additional Housekeeping Add Service _____	_____	\$ _____	Discontinue Effective _____
Pendant Add Service _____	_____	\$ _____	Discontinue Effective _____
Other _____ Add Service _____	_____	\$ _____	Discontinue Effective _____

Assisted Living or Garden House Changes

Prior LOC _____ New LOC _____ Effective Date _____

Prior Rate \$ _____ New Rate \$ _____ Effective Date _____

I authorize Merrill Gardens to make these changes in services and understand that I will be responsible for charges effective as shown above.

Resident

Date

Responsible Party

Date

Prepared BY (Merrill Gardens Team Member

Date



Definitions of Evaluation Terminology

Cognitive Guidance

If a resident who has some cognitive issues (for example: minimal short-term memory 20 pts and minimal long-term memory 20 pts) due to the diagnosis MCI should they be on care level one?

Conclusion: Yes, they should be on a care level one since we have to monitor their safety. MCI is a hot topic and we need to have the appropriate protections in place.

Guidance:

The current tool asks more specific questions and breaks the topic down into 3 components; orientation, short and long-term memory.

The type of impairment makes a difference in terms of how it may impact a resident's daily functioning and need for supervision.

- *Residents with orientation impairment may need regular redirection, reassurance and reminders for day to day tasks and activities. Depending on how severe the impairment, they usually can function in the general population.*
- *Residents with short term memory impairment frequently do not have insight into their limitations and may attempt to do more than they are able, leading them to put themselves in risky situations, such as going to the store, when they have no idea how to get home. MCI would require our team members to put interventions in place to observe for safety issues and supervisory needs, especially regarding ability to leave the community unsupervised. Depending on the individual, some may be able live with the general population, but others may need the extra protection of a memory care unit. Residents with significant short-term memory deficits may actually feel more comfortable in a memory care unit where they have a more manageable environment, a routine, and staff to cue and assist as needed.*
- *Residents with long term memory deficits in our setting typically are further along in their disease process and may also have short term memory and/or orientation deficits. Residents with long term memory deficits may not recall family members, or what age they are. They may see themselves as much younger with more abilities and duties. They commonly lack safety judgement and generally need to reside in a memory care unit to permit more independence.*

It is common for residents with neurocognitive issues to fluctuate in their self-awareness and abilities. Due to the significant safety risks, this is one area we should err on the side of caution in regard to ability to leave the community unsupervised.

Terminology throughout Evaluation

Occasional: Up to 25% of the time. At infrequent or irregular intervals; now and then.

Regular: 100% of the time. With a constant pattern, habitual, usually, or normal pattern of behavior.

Frequent: Up to 75% of the time. A habitual pattern of behavior but not consistently done.

Independent: The resident is able to complete/perform the task safely without modifications and within a reasonable amount of time. Resident required no assistance from staff. Resident able to perform task safely, alone, and secure. Note: may need some type of assistive device ex. walker but continues to be independent with the activity of daily living task.

Minimal/Setup Supervision: The resident is able to complete 75% of the activity of daily living task with staff completing 25% of the task. Resident requires a small amount of assistance from the staff such as cueing, touching, coaxing, and set up of required supplies. The residents' efforts while doing the task may not be done safely all of the time and requires staff supervision.

Moderate Assistance: The resident is able to complete 50% of the activity of daily living task with staff completing 50% of the task. Resident requires assistance with increased guidance and structure by the staff such as for balance, stability, and support to complete task. Resident may not be safe to complete task alone. Extra time may be required depending on the cognitive and functional level of the resident.

Extensive Assistance: The resident completes less than 50% but more than 25% of the activity of daily living task and the staff completes the remaining 50% to 75% of the task. Resident requires comprehensive support, guidance, and physical assistance to complete task. Resident is not safe to complete task alone. Extra time may be required depending on the cognitive and functional level of the resident.

Total Assistance: The resident completes less than 25% of the activity of daily living task and the staff completes up to 100% of the task. Resident is unable to initiate the task and requires total support, guidance, and physical assistance to complete task. Extra time may be required depending in the cognitive and functional level of the resident.



Immunization Record for: *Michael James Smith*

INFLUENZA (FLU) VACCINE

DATE	TYPE	LOT #	EXP DATE	GIVEN BY

PNEUMONIA VACCINE

DATE	TYPE	LOT #	EXP DATE	GIVEN BY

SHINGLES VACCINE

DATE	TYPE	LOT #	EXP DATE	GIVEN BY

OTHER VACCINES

DATE	TYPE	LOT #	EXP DATE	GIVEN BY



Tuberculosis Screening for:

TB SKIN TEST

DATE	TEST	LOT #	EXP DATE	RESULTS	READ BY

CHEST X-RAY

DATE	TYPE	LOT #	EXP DATE	GIVEN BY

1.3 Negotiated Service Agreements

.....Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Merrill Gardens recognizes and supports residents' rights to make choices about their care. We realize that some of their choices may not be in their best interest or according to their physician's orders. Part of the evaluation and service planning process includes identification of risk in the areas of falls, skin breakdown and elopement. In an effort to support resident choice and minimize risk in these areas, a Negotiated Service Agreement is to be used in a cooperative effort to inform the resident, family and physician (of Assisted Living residents) of the identified risk, and document agreed upon interventions to reduce the risk. The Negotiated Service Agreement is considered to be an extension of the Evaluation and Service Plan for Assisted Living residents.

WA: The Negotiated Service Agreement referred to in this policy is not to be confused with the broader term "negotiated service agreement" as used to refer to the Service Plan in WA regulations.

Definitions

Elopement

Exiting the community without following community procedures and the likelihood of inability to return unassisted.

Skin Breakdown

Stage I-IV pressure injuries.

Guidelines

Evaluation

The Evaluation is used to determine whether a resident is at risk for falls, skin breakdown or elopement.

- A score of 0-1 on the evaluation indicates the resident does not appear to be at risk.
- A score of 2-5 on the evaluation indicates the resident may be a low potential risk.
- A score of 6-9 indicates the resident may be a moderate potential risk.
- A score of 10-26 indicates the resident may be a high risk.

Physician Notification of Resident Condition

The Assisted Living resident's primary physician is informed and involved by use of the Physician Notification of Resident Condition form. When the risk is identified, this form is to be completed and sent to the physician. The Negotiated Service Agreement does not need to be completed prior to

notifying the physician. The top portion of the form is to be completed by the Community representative prior to sending. The physician is to check one of the two sections on the lower half of the form, write any orders they deem necessary, and sign and date the form before returning it to the Community. If the community has not received the completed form back, they are to contact the physician's office to request its return. The Community is to follow up on any orders, then file the form in the resident chart with physician orders.

Negotiated Service Agreement

Residents identified to be at-risk for falls, skin breakdown or elopement, are to have a Negotiated Service Agreement form completed. Appropriateness for residence in the community must also be considered. The Negotiated Service Agreement is an extension of the Service Plan for Assisted Living residents and is kept with the current Service Plan. The Negotiated Service Agreement is to be reviewed any time the Service Plan is reviewed and updated when a new Service Plan is completed.

To complete the Negotiated Service Agreement form:

- Check the appropriate box or boxes to indicate why the agreement is being completed.
- List the interventions or practices the community has put into place specifically for this resident to reduce risk on the form. The general rule when planning interventions is to KEEP IT SIMPLE. Interventions that are complicated or unrealistic are difficult to implement and maintain. One good simple intervention is usually more effective, and likely to be sustainable, than several complicated ones.
- Interventions may include resident, family or outside provider participation.

Acknowledgement and Agreement

The Negotiated Service Agreement is to be discussed with the resident and their family/responsible party, including the identified risk areas and specific interventions to reduce the risk. If the resident or family/responsible party does not want to participate in recommended interventions and a plan cannot be negotiated, indicate their preference on the form. This will provide documentation that they have been informed of an identified risk, and they have made a choice to accept that risk. The resident, family member/responsible party and the Community representative must all sign the Negotiated Service Agreement.



Negotiated Service Agreement

Resident:

This agreement documents the agreement between the Community and [name of family representative] regarding the care of the above-named resident. This agreement provides formal notification that the resident listed above:

- ☐ Has been determined to be at risk for falls
- ☐ Has been determined to be at risk for elopement
- ☐ Has been determined to be at risk for skin breakdown
- ☐ Has actual skin breakdown
- ☐ Other:

Your family member has been evaluated by the Community and is at risk for injury, which may cause permanent disability or be life threatening. The following interventions/practices are recommended to enhance the safety of your family member: [Indicate preference by checking box and initialing]

☐ accepted ☐ declined :

☐ accepted ☐ declined :

☐ accepted ☐ declined :

☐ accepted ☐ declined :

I have been given the opportunity to discuss my family member's condition and these interventions with the Community staff. I understand that these injuries may have severe consequences, and that the intervention(s) listed above may lessen the risk for an injury. However, I understand that the community and Merrill Gardens can make no guarantee that my family member will not injure themselves. I understand that assisted living programs are specifically designed to offer residents more independence and autonomy, and that 24-hour care and supervision is not provided on an individual basis. I acknowledge and agree that these interventions are not without risk and are impacted by the choices my family member makes.

Resident: _____ **Date:** _____

Family/Responsible Party: _____ **Date:** _____

Community Representative: _____ **Title:** _____



Evaluations and Service Planning

Resident Care Manual

1.4 Smoking.....Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Merrill Gardens is committed to providing a workplace and community living environment that promotes the health and well-being of its employees and residents. Consistent with that commitment, smoking is prohibited in all areas of the Community.

Definitions

Smoking

Use of any product that is burned or vaporized. Includes, but is not limited to cigarettes, cigars, pipes, battery operated or USB-charged cigars and cigarettes, electronic cigarettes, pipes that simulate smoking, and vaporizing devices.

Guidelines

Smoke-Free Environment

The Community has a non-smoking policy that is strictly enforced. Residents and their guests may only smoke in designated outdoor areas. Team members may not smoke with the residents. This policy will be firmly, but tactfully enforced.

WA: Smoking is prohibited within 25 feet of any common area, entrance or exit, window or intake vent. If there is no appropriate area that is at least 25 feet from the building(s), smoking will not be permitted on the property. Signage must be clearly posted at the community entrance.

Safety

All residents who smoke will be evaluated as a part of their initial Evaluation and subsequent re-evaluations using the EHR Evaluation. Those residents needing supervision or assistance will have this indicated on their Service Plan.



Evaluations and Service Planning

Resident Care Manual

1.5 Swimming PoolRevised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Some Merrill Gardens communities have a swimming pool for use by residents. The pool may be used for recreational and therapeutic activity. Reasonable effort will be made to ensure the pool area is safe, but Merrill Gardens cannot assume responsibility for accidents or injury that may occur from use of the pool.

Guidelines

Security

The pool area in licensed communities will be kept secured from unauthorized access by means of a locking door or gate. There is no lifeguard on duty. If supervision is required or available, hours will be posted at the entrance to the pool area. Residents choosing to utilize the pool unsupervised, do so at their own risk.

Screening

All residents receiving assisted living services or residing in a licensed apartment desiring access to the pool area, will be screened for ability to use the pool independently. The Mini Mental Status Exam will be utilized as a basis to determine whether a resident is permitted access to the pool area. A score of 24 or less may indicate cognitive impairment and the resident will not be granted unsupervised access to the pool area. If a resident has a significant change in condition, they will be rescreened for their ability to access the pool area. Please note on the Service Plan if the resident is granted unsupervised access to the pool area.

Pool Rules

The following rules are in effect at Community pools:

- The Community reserves the right to prohibit access to anyone for any reason.
- Residents and their guests use the pool and surrounding area at their own risk.
- Residents must accompany guests and remain responsible for their behavior and any damages or injury.
- Residents must supply their own towels and are responsible for cleaning up after themselves and their guests.
- Hours of supervision will be posted.
- Running, pushing, wrestling or causing undue disturbance in or around the pool area is prohibited.

- Proper swimming attire is required. No street clothes will be allowed in the pool.
- Any child or adult who wears diapers or incontinence briefs must use a product intended for swimming.
- Devices that play music are allowed only with earphones, unless part of an approved activity.
- Glass is not permitted in the pool area.
- Beverages, food or refreshments brought to the pool area must be in non-breakable containers.
- Persons having any abrasions, open blisters, cuts, sores, etc. are not allowed in the pool.
- Persons with eye inflammation, colds, nasal or ear discharge or any infection may not use the pool.
- Animals, other than service animals, are not allowed in the pool area.
- Persons having currently active diarrhea or who have had active diarrhea within the previous 14 days shall not be allowed to enter the pool water.

Activities

The pool and surrounding area may be used for Community and therapeutic activities. These events are planned and will be posted on the activity calendar. Planned activities take priority over recreational use of the pool and surrounding area. The pool and surrounding area may be closed to general use during these times.



Evaluations and Service Planning

Resident Care Manual

1.6 Respite.....Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

The Community may provide respite services to provide temporary relief from the demands of caretaking, contingent upon management decision. Respite services may be provided at a daily rate, but generally should not exceed 30 days for any single stay.

To the extent applicable, recipients of respite services are subject to the same policies and procedures as other residents residing in the Community.

SC: Respite care must be for a period of more than twenty-four hours, but less than six weeks.

WA: Respite stays are limited to thirty days or less.

Guidelines

Philosophy & Program

Prospective residents and their responsible party may want to take advantage of this option for several reasons:

- He or she is thinking about moving into a Community, but wants to stay for a short time before deciding.
- He or she is being discharged from a hospital or nursing home, but may still need some extra help to ease the transition home.
- A family member or caregiver is going away on vacation, and their loved one needs a temporary place to live.

Respite residents will have access to the same services available to all residents, including a furnished room, meals, scheduled transportation, activity programs and care as designated in the Service Plan.

Evaluation and Service Plan

The Licensed Nurse or designee will complete an Evaluation and Service Plan for each Resident.

AZ: The Evaluation and Service Plan will be completed no later than three working days after the resident's date of acceptance. If the resident has a Service Plan in the Community record that was developed within the previous 12 months, it will be reviewed and updated within three working days after the resident's date of acceptance. If the resident is not expected to be present in the Community for more than seven days, they do not need to provide evidence of freedom from infectious Tuberculosis.

WA: The Community must obtain sufficient information about the individual and any needed physician orders for diet, medications and care to meet their anticipated needs. The EHR Evaluation and Service Plan is completed at move-in.

Post-Respite Summary

At the end of the respite period, Community team members may provide a summary that includes a copy of the resident's Service Plan and other information that might assist the resident or their responsible party with ongoing needs or transition back home.

Respite Resident Agreement

Respite residents are required to complete the respite resident admissions packet including a signed Residency Agreement appropriate for the community's state and license type.

Advance Directives for Respite Residents

Team members will be informed of any individuals with an advanced directive. A copy of any advanced directives should be filed in the respite record.